Online Appendix

Roll-Ups and Mark-Ups: Evidence on Extractive Takeovers in Veterinary Care

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A Background

A.1 The two chains and the Swedish rules

The market. Before 2011, the Swedish market was highly fragmented, with mostly small, owner-run clinics. About 31% of Swedish households have a dog or a cat (Agria, 2026), and cats and dogs account for the bulk of the market. The real revenue of veterinary clinics has grown much faster than the number

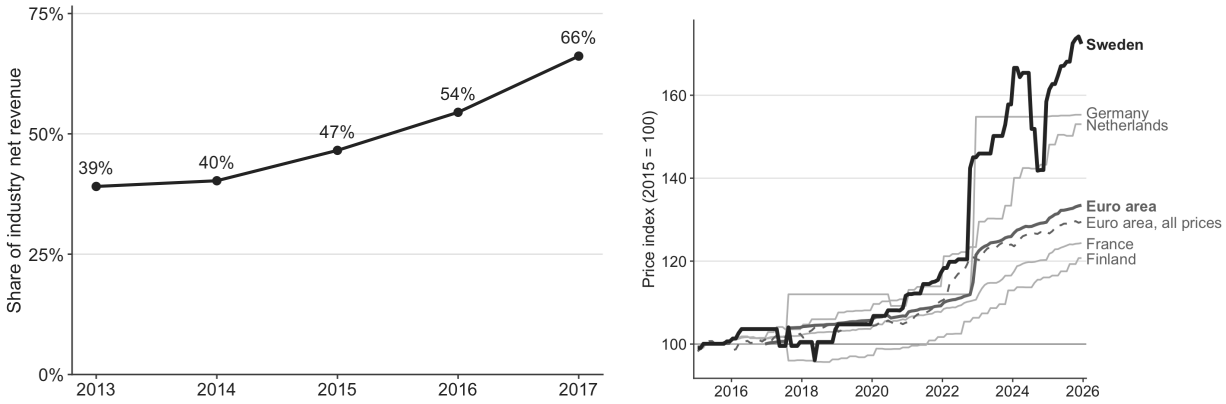
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of registered dogs (Figure A.2(b)), while the number of cats rose by about 33% between 2006 and 2024 (Statistiska centralbyrån (SCB), 2013; Novus, 2024).

AniCura. AniCura was founded in 2011 by the private equity firm Fidelio Capital, through a merger of five clinics and hospitals in Sweden and Norway. After 60 add-on acquisitions, Fidelio sold it to Nordic Capital in 2014. Under Nordic Capital, AniCura grew by 46% a year on average and had 250 clinics in 2018, when Mars Petcare acquired it (Valjevac and Røren, 2019), reportedly at a seven-fold return to Nordic Capital (Private Equity News, 2018).

Evidensia. Evidensia was founded in 2012 by the private equity firm Valedo, through a platform acquisition of several larger hospitals and clinics followed by add-on acquisitions. EQT became the majority shareholder in 2014, and in 2017 Evidensia merged with the British group IVC to form IVC Evidensia. Nestlé (2019) and Silver Lake (2021) became minority investors; the 2021 investment valued the group at about EUR 12.3 billion, and EQT remains the largest shareholder (EQT, 2021). In 2023–2024, the Swedish entity made group contributions of about SEK 636 million to its parent company in Luxembourg (Cervenka, 2026).

Price transparency and merger review. The Swedish competition authority finds "significant problems with insufficient price transparency vis-à-vis animal owners" [author's translation] (Konkurrensverket, 2026). Mergers below the notification thresholds can be called in: if the firms together have a Swedish turnover above SEK 1 billion, but fewer than two of them exceed SEK 200 million, the authority may order notification for special reasons (*Konkurrenslagen*, Ch. 4, § 7). The chains exceed this level, but the authority rarely uses this power.

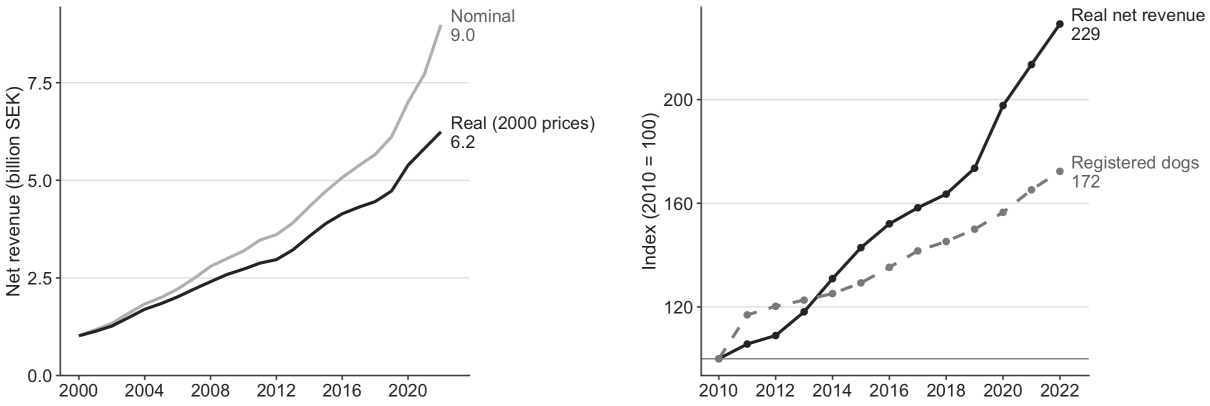


(a) Share of industry net revenue

(b) Price changes since 2015, European countries (Eurostat)

Figure A.1: The chains' share of the market, and prices across Europe

Notes: Measure: (a) the net revenue of Evidensia and AniCura in Sweden as a share of the net revenue of all veterinary clinics (Figure A.2(a), nominal); (b) the harmonised index of consumer prices (HICP) for veterinary and other services for pets (ECOICOP 09.3.5), 2015 = 100, with the all-items HICP of the euro area for comparison. *Sample:* (a) 2013–2017 (AniCura's Swedish accounts are not available after 2017); (b) monthly, January 2015 to December 2025 (the euro-area series for veterinary services starts in December 2016). *Source:* (a) annual reports of the Swedish companies and Statistics Sweden; (b) Eurostat (ECB Data Portal).



(a) Net revenue, nominal and real

(b) Registered dogs and real net revenue

Figure A.2: Net revenue of veterinary clinics in Sweden and the number of registered dogs

Notes: Measure: (a) the net revenue (net turnover excluding excise duties) of veterinary clinics in Sweden (industry code SNI 75), billion SEK, nominal and real (deflated by the consumer price index, 2000 prices); (b) real net revenue and the number of registered dogs, index 2010 = 100. *Sample:* (a) 2000–2022, over which real net revenue rose 6.1-fold; (b) 2010–2022; in 2022 the index is 229 for revenue and 172 for registered dogs; part of the early rise in registered dogs (17% in 2011) reflects the register filling up. *Source:* Statistics Sweden, Structural Business Statistics (*Företagens ekonomi*) (Statistics Sweden, 2026); the dog register of the Swedish Board of Agriculture (Swedish Board of Agriculture, 2025).

B Data

B.1 The data

Coverage. The data include only clinics with a digital invoice system, which excludes very small clinics with paper invoices; coverage of Swedish clinics in 2025 is around 78%. Clinics and insurers adopted electronic claims only recently, so the data are concentrated in recent years (Figure B.1(a)), which limits the number of acquisitions with enough years before and after.

Diagnosis. Diagnosis codes follow the hierarchical classification *Pyramidion*, which allows aggregation: specific codes such as "DA.02.03.01" and "DA.02.01.01" are grouped into the *diagnosis type* "DA.02".

Breeds. The sample includes only dogs and cats. The many breeds are grouped into 15 LLM-generated *breed types* (e.g., "Dog – Retriever"); results are robust to the full set of breed fixed effects.

Regions. Clinic locations are observed by municipality. The regressions use region fixed effects (21 regions) and a "City Type" fixed effect: the four largest cities, "Large" ($\geqslant$ 100k inhabitants), "Medium" (30k–100k) and "Small" ($<$ 30k). Results are qualitatively robust to municipality fixed effects, at the cost of lower precision.

Table B.1: From the data to the samples

Step	Visits	Clinics	Used for
Panel A: price comparisons			
1. All visits in the claims data	451,689	533	
2. Without visits at clinics owned by Svenska Veterinärgruppen	430,825	531	
3. Clinics with a visit in each of 2022–2025 and at least 10 visits	397,894	312	Spending per visit on all visits (Table C.1)
4. Visits with invoice lines, at least one with a positive price	336,819	312	Basket indices, spending per visit (Table 1); line prices (Table C.1)
Panel B: effect of acquisition			
1. Clinics never owned by Svenska Veterinärgruppen, with a visit in each of 2022–2025	390,522	289	
2. Without clinics owned by a chain throughout; visits from 2017/18; at least 10 visits	176,976	204	
3. Acquired clinics with visits in at least two years before and one year after the acquisition year	172,259	200	Effect of acquisition

Notes: Measure: the visits and clinics left after each rule; each row applies one more rule to the row above. *Sample:* Panel A, the price comparisons; Panel B, the effect of acquisition; both start from the same data. Panel A drops the visits at clinics owned by Svenska Veterinärgruppen while the chain owns the clinic; visits without reported invoice lines have a total but no lines, so in the price comparisons they enter only spending per visit on all visits (Table C.1). Panel B drops the clinics ever owned by Svenska Veterinärgruppen; the main DiD sample has 12 acquired and 188 never-acquired clinics. Row 3 of Panel A and row 2 of Panel B also drop visits with zero spending. *Specification:* basket price indices have one observation per clinic and year with enough priced services (Table 1); regressions can drop a few observations that are alone in a fixed-effect cell. *Definitions:* Section B.2.

Table B.2: Summary statistics: the data in numbers

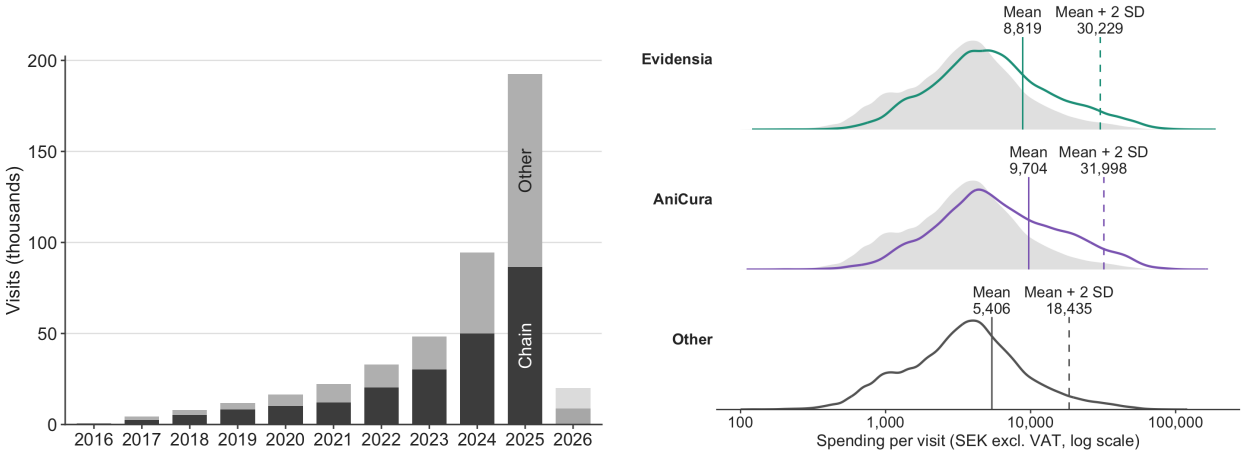
Item	Number
Visits	451,689
Invoice lines	3,055,765
Clinics	533
Insurers	4
Years covered	2015 to 2026
Distinct invoice texts (trimmed)	78,665
Services the texts are assigned to	685
Item categories	26
Diagnoses	7,606
Breeds	4,115

Notes: Measure: counts of visits, invoice lines, clinics and the insurers that supplied the claims; the distinct invoice texts after trimming (Section B.3); the services to which they are assigned (7 Core and 678 catalogue services, Section B.4; the services of the baskets: Table B.8); the 26 item categories that one insurer's experts assigned to invoice texts, extended to the other texts (Section B.3); and the distinct primary diagnosis codes and breed names. *Sample:* all visits in the data, at all clinics, 16 December 2015 to 10 February 2026.

Table B.3: Summary statistics: clinics by ownership group

Ownership group	Clinics	Visits	Share of visits	Share of revenue	Spending per visit (SEK): median	Mean	SD
Other chain clinics	100	219,493	0.486	0.608	5,268	9,141	10,878
Acquired	12	22,691	0.050	0.051	4,248	7,446	9,335
Never acquired	393	181,162	0.401	0.295	3,596	5,367	6,425
Svenska Veterinärgruppen	28	28,343	0.063	0.046	3,440	5,363	6,784
Total	533	451,689	1	1	4,311	7,305	9,204

Notes: Measure: clinics, visits, the shares of visits and of revenue (total spending over all visits, excluding VAT), and spending per visit (the total billed for a visit, excluding VAT, not winsorised), by ownership group. *Sample:* all visits in the data; each clinic is in one row with all its visits, so Acquired and Other chain clinics include the visits before a chain bought the clinic. Acquired: the 12 clinics acquired by AniCura or Evidensia during the data and used to estimate the effect of acquisition; other chain clinics: all other clinics with visits under AniCura or Evidensia ownership; never acquired: clinics never owned by AniCura, Evidensia or Svenska Veterinärgruppen; Svenska Veterinärgruppen: clinics ever owned by it. *Definitions:* Section B.2.



(a) Observations by year

(b) Spending per visit (pre-winsorised), by owner

Figure B.1: Observations in the claims data and the density of spending per visit

Notes: Measure: (a) visits per calendar year; (b) the density of spending per visit (the sum of the visit's invoice lines excluding VAT, before winsorisation) on a log scale, with the mean of the group (solid line) and the mean plus two standard deviations (dashed line), both computed in SEK. *Sample:* (a) all visits in the claims data, at clinics owned by Evidensia or AniCura (Chain) and at all other clinics (Other); years with fewer than 100 visits (2015) are not shown, and 2026 is a part year (the data end on 10 February 2026); (b) visits of at least 100 SEK, by owner; the grey area in the Evidensia and AniCura panels repeats the density of the other clinics.

B.2 Definitions and samples

The notes to the tables and figures refer to the following definitions.

Basket price index. See Section B.6.

Never-acquired clinics. Clinics never owned by AniCura, Evidensia or Svenska Veterinärgruppen in the data, and not one of the 12 acquired clinics.

Independent clinics. Clinics not owned by AniCura, Evidensia or Svenska Veterinärgruppen, including Distriktsveterinärerna (the state-run district veterinary service) and a few much smaller chains.

Ownership. Acquisitions are identified from the name changes that AniCura and Evidensia always make, checked against financial statements and online sources. Most independent clinics are run by an owner with no other clinic.

Sellers. The 12 acquired clinics were sold by five companies (clinic groups A–E, Table B.4): groups A and B sold five and four clinics in one acquisition each, and groups C–E one clinic each. All five sellers were controlled by local owners. Online sources (company registers, annual reports, clinic websites and press) indicate that each was owned, at least in part, by a veterinarian, and in at least three cases the veterinarian kept working at the clinic after the sale. Before its sale to Evidensia, group A's holding company was owned 51% by a company of its local owners, which consolidated it, and 49% by AniCura. Leaving out group A does not weaken the result (Figure D.2).

Table B.4: Timing of the acquisition and of the IT switch for each acquired clinic

Clinic	Clinic group	Acquirer	Closing	First treated year	IT switch	Months to switch	Hospital	Main DiD	No common ownership
Clinic 1	Clinic group A	Evidensia	2022-02-01	2021/22	not detected			yes	yes
Clinic 2	Clinic group A	Evidensia	2022-02-01	2021/22	2024-02	24		yes	
Clinic 3	Clinic group A	Evidensia	2022-02-01	2021/22	2024-02	24	yes	yes	
Clinic 4	Clinic group A	Evidensia	2022-02-01	2021/22	2024-01	23	yes	yes	
Clinic 5	Clinic group A	Evidensia	2022-02-01	2021/22	2024-01	23		yes	yes
Clinic 6	Clinic group B	Evidensia	2022-09-30	2021/22	2023-04	7		yes	
Clinic 7	Clinic group B	Evidensia	2022-09-30	2021/22	2023-04	7		yes	
Clinic 8	Clinic group B	Evidensia	2022-09-30	2021/22	2023-05	8		yes	
Clinic 9	Clinic group B	Evidensia	2022-09-30	2021/22	2023-04	7		yes	yes
Clinic 10	Clinic group C	Evidensia	2023-10-01	2023/24	2024-03	5		yes	yes
Clinic 11	Clinic group D	AniCura	2023-11-30	2023/24	2024-04	5		yes	yes
Clinic 12	Clinic group E	Evidensia	2024-03-01	2023/24	2024-11	8		yes	yes

Notes: Measure: for each acquired clinic, the closing date of the acquisition, the first treated year (October to September, the year that contains the closing date), the month of the IT switch and the months from the closing to the switch. *Sample:* the 12 acquired clinics, anonymised and numbered by closing date; a clinic group is the clinics sold by the same company in one acquisition; no common ownership marks the acquisitions in which the acquirer owns no other clinic in the city at any time in the data (the sample of Figure 2 and Table F.1). *Definitions:* Section B.2 (IT switch, no common ownership).

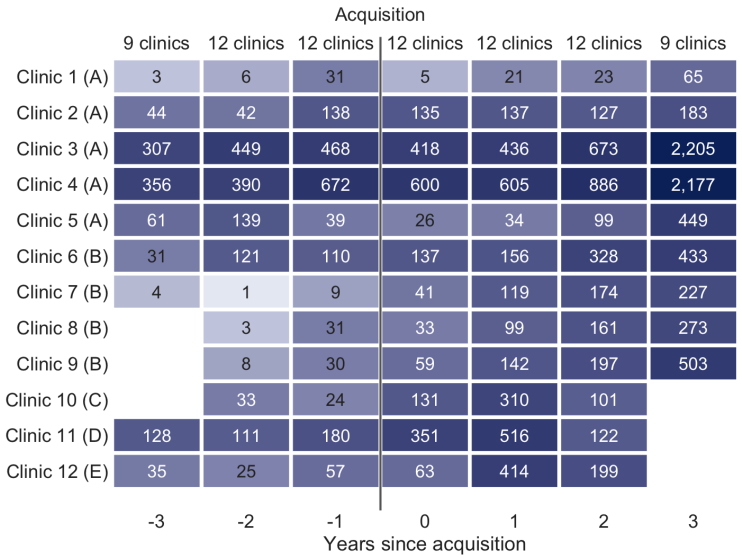


Figure B.2: Acquired clinics by year since acquisition

Notes: Measure: for each acquired clinic, whether it is observed in a year since acquisition (a tile) and its visits in that year (the number and the shade); above each column, the number of acquired clinics observed; vertical line: the acquisition. *Sample:* the 12 acquired clinics, numbered by closing date with their clinic group in brackets (Table B.4); 9 were first treated in 2021/22 and 3 in 2023/24, so the clinics acquired last are not yet observed in the last years since acquisition. Years run from October to September.

Chain dummy. Equal to one in the years in which the clinic is owned by AniCura or Evidensia; in the calendar year in which the owner changes, equal to one if the chain accounts for the majority of the clinic-year's price observations (service-months). In the regressions on invoice lines and visits, ownership is measured at the visit.

Samples. Cross-sectional sample: visits at clinics with a visit in each of 2022–2025 and at least 10 visits, without visits under Svenska Veterinärgruppen, whose acquisitions are too recent to study. Main difference-in-differences sample ("main DiD sample"): the acquired and the never-acquired clinics, hospi-

tals included, from 2017/18, with the same clinic rules; an acquired clinic enters only with visits in at least two years before the year of its acquisition and one after it, which gives the 12 acquired clinics; clinics owned by a chain throughout are not in the sample. A few much smaller chains count as independent. Table B.1 gives the visits and clinics after each step.

Event time. Years in the difference-in-differences estimates run from October to September. Event year 0 is the year that contains the closing date of the acquisition, so the clinic is owned by the acquirer for only part of year 0; year -1 is the reference year.

IT switch. The first month in which at least 25% of the clinic's invoice lines carry invoice texts specific to the acquirer. It is measured on the non-emergency invoice lines of one insurer. An invoice text is specific to a chain in a year if it is billed by at least six other clinics of Evidensia (four of AniCura) and by at most 5% of the never-acquired independent clinics. A clinic-month is used if it has at least five invoice lines, and the switch is the first month from January 2019 in which the share is at least 25% and is at least 25% again in at least one of the next two observed months.

No common ownership. A city is a municipality. An owner has common ownership in a city if it has more than one clinic there at any time in the data; the definition is by clinic and owner and does not vary over time. In the difference-in-differences estimates without common ownership, a clinic is dropped if its owner, as named in the clinic register, has another clinic in the city; independent owners are not named, so only acquisitions by a chain that owns another clinic in the city are dropped.

Spending per visit. The sum of the visit's invoice lines, excluding VAT; for the visits that one insurer reported without invoice lines (almost all in 2025), the total of the claim.

Winsorisation. Spending per visit and invoice line prices are winsorised at the 1st and 99th percentiles. Results are robust to other winsorisation levels of prices.

Item category. One of 26 broad categories, from the insurer's experts or predicted from the text (Section B.3).

Estimators. TWFE: event study with clinic and year fixed effects and base year -1; treated clinic-years outside events -3 to $+3$ are dropped; the reported estimate is the mean of events 0 to 3, weighted by the acquired clinics in each event year, and the pre-trend test is a joint test of the leads -3 and -2. CS: Callaway and Sant'Anna (2021), never-acquired clinics as controls (never treated), universal base -1, doubly robust; the reported estimate is the average of events 0 to 3, and the pre-trend test is a joint test of events -3 and -2. dCDH: de Chaisemartin and D'Haultfœuille (2020), never-acquired clinics as controls (never switchers); the reported estimate is the average total effect of events 0 to 3, and the placebo test is a joint test of the placebos -3 and -2. When the event is dated by the IT switch, the last event year is $+2$. Standard errors are clustered by clinic unless a note says otherwise. In CS and dCDH, the controls are the never-acquired clinics. In TWFE, the year effects are estimated from all clinics in the sample (which excludes clinics owned by a chain throughout), so acquired clinics in other event years also serve as implicit controls.

B.3 Broad categories

Experts at one insurer have placed about 22,500 distinct invoice texts into 26 broad categories, such as "Lab tests" and "Diagnostic imaging". The other 125,000 texts get a category in three steps. Each text is first *trimmed* (lower-cased and stripped of numbers, units and punctuation: "Onsior inj 20 mg/ml" becomes "onsior inj"); about 43,000 then equal an expert-categorised text and get its category. The remaining 64,500 trimmed texts are converted into embeddings, vectors that place texts with similar meaning close

together, by the Swedish sentence-BERT model of the National Library of Sweden (KBLab, 2021): their Swedish BERT model (Devlin et al., 2019; Malmsten et al., 2020) trained to give sentence embeddings by the method of Reimers and Gurevych (2020). The five nearest expert-categorised neighbours vote: a category is assigned if the two nearest neighbours share it or if at least three of the five do; otherwise fixed rules merge closely related labels and the vote is repeated, and texts without a majority remain unclassified. Finally, keyword rules assign a category from words in the text (e.g., texts with "prov" ('sample') go to lab tests), also for lines that the experts categorised. In a leave-one-out test on the 16,808 trimmed expert texts, 92% of texts receive a category and 82% of these get the experts' category (86% of invoice lines).

B.4 Classification of services

The goal is to compare the price of *the same service* across clinics: two invoice texts belong to the same service if a clinic could list them as one item in its price list. The texts are grouped into services in five steps. In the steps done by AI agents, each agent works independently on anonymised "cards": the invoice text with company, clinic and place names masked, how often it is billed, and what it is billed together with, but no prices, dates, clinics, owners or insurers. Each step combines the agents by a rule fixed in advance and is checked against texts whose answer is known.

Step 0. Written text rules define the seven core services (fees and simple tests such as the prescription fee, the blood sampling fee and a CRP test). They are fixed services in Steps 2 and 3. In the robustness checks, the Core services instead include 24 texts that the rules leave out but the review of Step 4 placed in a Core service (incl. 24 contested texts), consist only of the texts listed when the rules were written (written rule), or leave out Core lines that name a second service or are billed under anaesthesia or with another procedure (stand-alone).

Step 1. The 13,625 texts with at least five invoice lines but no expert category get one of the 26 categories by a majority of three runs of Claude Opus 5.5. On 1,000 hidden expert-categorised texts, the runs give the experts' category for 70% (80% with neighbouring categories merged). This category only decides to which of four fields a text is sent in Steps 2 and 3. It is independent of the BERT categories, and the two agree well: for 74% of the invoice lines that BERT classifies (87% with related categories merged), and across all lines only 3% change category (Table B.5). The line-price premium is 34% with these categories instead of 33% with BERT (Table C.1).

Step 2. The 2,869 frequent texts (at least 50 invoice lines; 82.5% of the lines in service categories) are sorted within four fields by six independent runs, three with Claude Opus 5.5 and three with Claude Sonnet 5.5. Each run defines services by content and unit, and assigns every text to a service or excludes it (generic texts, part-charges, packages, items priced by time or dose, combinations, goods, unclear texts). For example, "X-ray, 1–2 images" and "X-ray, 3–4 images" are separate services, while the same analysis is one service whatever the analyser. A service is of *grade A* if its unit is bounded and it is a standard, narrow item that is the same for every patient (a single analyte, a named panel or a fee), of *grade B* if the unit is bounded but another of these fails (a consultation or sedation), and of *grade C* if the unit is open. Texts are grouped by a vote in which both models carry equal weight; a service's grade is the majority grade within each model, then the stricter of the two.

Step 3. The less frequent texts (10 to 49 invoice lines) are matched to the frozen catalogue of Step 2 by six new runs; a text gets the service given by at least four of them. Of 300 hidden Step-2 texts, 92.7% get exactly the right service and 0.7% a different one.

Step 4. I review the catalogue by written rules fixed before the review files were opened, without seeing prices or estimates: 41 merges of services that differ only in wording, field or analyser, and 33 changed

Table B.5: Agreement between the BERT and the LLM categories

	Unique invoice texts	Invoice lines
Panel A: all texts without an expert category		
Same category	69.9	71.6
Same category, related categories merged	82.2	83.8
N	13,625	248,577
Panel B: the texts that BERT classifies		
Same category	73.5	74.2
Same category, related categories merged	86.4	86.9
N	12,956	239,840
Panel C: all invoice lines, by the source of the category		
The insurer reports the category		8.1
Text identical to a text that the insurer's experts categorise		70.1
Text sent to the LLM agents		14.2
Text categorised by BERT only		7.1
No category		0.5
Category differs between BERT and the LLM agents		3.3
N		3,055,765

Notes: Measure: Panels A and B, the share (%) of invoice texts, and of their lines, with the same category under BERT and under the LLM agents (the category of at least two of three runs), also with related categories merged (lab and blood tests; examinations, re-visits, treatments and fees; surgery, anaesthesia and dental care; drugs, materials, food and retail goods; euthanasia and cremation; the residual categories); Panel C, all invoice lines by the source of their category. *Sample:* Panels A and B, the invoice texts with at least five lines and no expert category, and their lines (non-emergency visits with a positive price, January 2018 to February 2026, outside Svenska Veterinärgruppen); Panel A counts texts that BERT leaves uncategorised, or on which the LLM runs split or that they mark unclear, as disagreements, while Panel B keeps only the texts that BERT classifies and the LLM runs categorise; in Panel C, the texts categorised by BERT only are mostly texts with fewer than five lines. *Definitions:* Sections B.3 and B.4.

grades. The result is 685 services and the 5,866 invoice texts assigned to them, out of 78,665 distinct texts, covering 51% of all invoice lines (Table B.6).

Steps 2 and 3, the core services and the screen below were set after preliminary estimates had been seen; Figures C.1 and E.1 show the estimates for alternative maps, grades and baskets.

B.5 Pricing baskets

The *Core basket* consists of the seven core services. A graded service enters a basket only if, between January 2019 and March 2025, it is billed by at least five Evidensia clinics, five AniCura clinics and five never-acquired independent clinics (without Distriktsveterinärerna), so that its price can be compared across Evidensia, AniCura and independent clinics. The acquired clinics of the difference-in-differences are not counted, so that which services enter a basket does not depend on the prices the difference-in-differences estimates. The *Main basket* consists of the Core services and the screened services of grade A, and the *Broad basket* adds the screened services of grade B; services of grade C enter no basket. Table B.6 gives the size of each basket; results without the screen, or with a looser one, are in Table C.1 and Figures C.1 and E.1. Table B.8, in Section B.7, lists the services of each basket.

Table B.6: Invoice texts, services and baskets in numbers

	Core basket	Main basket	Broad basket	All services
Core services	7	7	7	7
Grade A services		63	63	197
Grade B services			64	477
Services, total	7	70	134	685
Invoice texts	563	2,043	3,205	5,866
Share of invoice lines (%)	16.2	21.8	35.9	51.2

Notes: Measure: the number of services in each basket, by grade, the invoice texts assigned to them and their share of invoice lines (% of all 3,055,765 lines); All services counts every service to which invoice texts are assigned, including 4 services of grade C, which are in no basket. *Sample:* the 78,665 distinct invoice texts; the texts not assigned to a service are goods, items priced by time or dose, generic texts, packages and rare texts. *Definitions:* grades, Section B.4; baskets, Section B.5; the services: Table B.8.

B.6 Price index

The price of a service at a clinic in a month is the modal price of its invoice lines in that month (non-emergency visits from January 2018, excluding VAT, rounded to whole kronor); if the service is billed more than once in the month, the modal price is used only if more than half of the lines carry it. Lines that are one part of a service billed in several lines are not priced. A clinic's price of a service in a year is the median of its monthly modal prices. The index of a clinic in a year is the mean, over the basket services that it prices in that year (at least three), of the log price minus the mean log price of the reference clinics, the never-acquired clinics, for the same service and year (at least three such clinics). In the price comparisons, years are calendar years (January 2019 to February 2026) and clinic-years are weighted by the number of services priced; in the difference-in-differences, years run from October to September and clinic-years are unweighted.

Comparing price measures. The rank correlation of clinic price levels is 0.86–0.94 across the three baskets, 0.79–0.82 between the baskets and the prices of all invoice lines within free text, and 0.63–0.70 between the baskets and the prices within broad category, with BERT or LLM categories, which rank clinics almost identically (0.996; Table B.7). The chain premium and the effect of acquisition are similar under every definition of the baskets (Figures C.1 and E.1).

Table B.7: Rank correlation of clinic price levels across the price measures

Measure	Clinics	Core	Main	Broad	Line price, category FE (BERT categories)	Line price, category FE (LLM categories)
Core	248					
Main	287	0.899				
Broad	306	0.860	0.942			
Line price, category FE (BERT categories)	312	0.626	0.668	0.700		
Line price, category FE (LLM categories)	312	0.627	0.665	0.703	0.996	
Line price, free-text FE	312	0.794	0.801	0.821	0.720	0.714

Notes: Measure: Spearman rank correlations across clinics of the clinic price levels under six price measures; a clinic's price level under a measure is its clinic fixed effect in the preferred specification of the measure (Table 1 and Table C.1), estimated without the chain dummy and without the clinic-level controls (hospital, region, city type). *Sample:* each correlation uses the clinics with a price level under both measures (Clinics: the number of clinics with a price level under the measure); a clinic that changes owner has one price level. *Specification:* category fixed effects use the experts' categories, extended to the other invoice texts by BERT or by LLM agents (Sections B.3 and B.4). *Definitions:* Section B.2.

B.7 The services in the baskets

Table B.8: The services in the three baskets

Service	Category	Grade	Basket	Evidensia	AniCura	Independents	Lines
Direct-settlement fee		Core	Core	62	33	155	126,803
Prescription fee		Core	Core	61	6	177	33,920
Blood-sampling fee		Core	Core	47	36	149	18,435
Subcutaneous injection		Core	Core	60	36	126	15,728
Haematology		Core	Core	59	35	179	11,479
CRP test		Core	Core	60	34	166	10,855
Euthanasia		Core	Core	59	36	113	4,105
Urinalysis, complete (dipstick + SG + sediment)	Provtagning/lab	A	Main	54	26	87	6,886
Bacterial culture with resistance testing	Provtagning/lab	A	Main	57	36	76	4,135
Total T4, single analyte	Provtagning/lab	A	Main	59	28	136	4,050
Chem 10 panel	Provtagning/lab	A	Main	41	18	78	3,716
Creatinine, single analyte	Provtagning/lab	A	Main	51	28	47	3,696
ALT (ALAT), single analyte	Provtagning/lab	A	Main	53	27	41	3,459
Fluorescein staining of cornea	Undersökning	A	Main	60	32	94	3,412
Electrolyte panel (Na, K, Cl / Lyte 4)	Provtagning/lab	A	Main	50	20	97	2,835
Histopathology, 1 site/organ/biopsy	Provtagning/lab	A	Main	46	17	71	2,301
Cystocentesis (urine sampling)	Provtagning/lab	A	Main	31	30	76	2,144
Pancreas-specific lipase (cPL/fPL), quantitative	Provtagning/lab	A	Main	51	16	60	1,962
Fructosamine, single analyte	Provtagning/lab	A	Main	53	25	90	1,916
Claw clipping	Kloklippning	A	Main	58	30	105	1,758
Otoscopy / ear examination	Undersökning	A	Main	48	15	60	1,648
Urine protein/creatinine ratio	Provtagning/lab	A	Main	53	20	38	1,565
IV injection, administration	Behandling	A	Main	44	10	46	1,550
ALP, single analyte	Provtagning/lab	A	Main	43	27	33	1,550
Blood pressure measurement	Blodprov	A	Main	48	27	77	1,499
Serum amyloid A (SAA)	Provtagning/lab	A	Main	38	15	54	1,486
Tonometry (intraocular pressure)	Undersökning	A	Main	47	30	39	1,463
Albumin, single analyte	Provtagning/lab	A	Main	31	17	6	1,380
SDMA, single analyte	Provtagning/lab	A	Main	47	22	79	1,363
Urine sediment	Provtagning/lab	A	Main	46	21	54	1,332
Anal sac flushing	Behandling	A	Main	59	33	84	1,240
Cobalamin (vitamin B12), single analyte	Provtagning/lab	A	Main	39	11	32	1,176
Bile acids, single sample	Provtagning/lab	A	Main	42	25	48	1,137
Urea, single analyte	Provtagning/lab	A	Main	42	26	31	868
Dental X-ray, first/one image	Bilddiagnostik	A	Main	55	17	25	868
cPL/fPL SNAP test	Provtagning/lab	A	Main	39	14	31	865
Cortisol, single test	Provtagning/lab	A	Main	42	23	34	856
Topical eye anaesthesia	Narkos/sedering	A	Main	42	26	37	852
Urine dipstick	Provtagning/lab	A	Main	23	15	46	840
Biomicroscopy / slit lamp examination	Provtagning/lab	A	Main	9	10	8	835
Total protein, single analyte	Provtagning/lab	A	Main	34	16	13	832
Cobalamin + folate panel	Provtagning/lab	A	Main	10	17	20	797
Vaccination DHPPi	Förebyggande beh	A	Main	57	6	57	774
X-ray, extra image (threshold not stated)	Bilddiagnostik	A	Main	5	25	32	768
Schirmer tear test	Provtagning/lab	A	Main	45	29	27	756
Anal sac emptying	Undersökning	A	Main	53	24	74	710
Vaccination Pi (canine parainfluenza)	Förebyggande beh	A	Main	56	5	48	709
Chem 15 panel	Provtagning/lab	A	Main	27	6	20	655
Urinalysis: dipstick + specific gravity	Provtagning/lab	A	Main	41	16	9	627
Phenobarbital, single analyte	Provtagning/lab	A	Main	46	18	19	617
Postage/dispatch fee	Administrativ avgift	A	Main	5	8	15	558
ACTH stimulation test	Provtagning/lab	A	Main	42	29	32	535
Urine specific gravity	Provtagning/lab	A	Main	33	15	36	527

Phosphate, single analyte	Provtagning/lab	A	Main	34	24	23	497
Glucose, single analyte	Provtagning/lab	A	Main	11	21	22	487
T4 + TSH profile	Provtagning/lab	A	Main	17	19	13	434
Platelet count	Provtagning/lab	A	Main	16	7	5	425
Total bilirubin, single analyte	Provtagning/lab	A	Main	21	12	12	380
Calcium (total), single analyte	Provtagning/lab	A	Main	23	15	18	331
Urinary stone analysis	Provtagning/lab	A	Main	30	23	23	323
Ionised calcium, single analyte	Provtagning/lab	A	Main	12	8	5	313
Triglycerides, single analyte	Provtagning/lab	A	Main	26	13	14	292
TSH, single analyte	Provtagning/lab	A	Main	23	11	25	286
Urine cortisol/creatinine ratio	Provtagning/lab	A	Main	37	18	19	271
Corneal debridement	Behandling	A	Main	18	13	7	223
Urinary catheterisation	Behandling	A	Main	19	11	17	184
Intra-articular injection	Behandling	A	Main	8	6	10	168
Tear duct flushing	Behandling	A	Main	23	17	24	159
ID microchipping	Förebyggande beh	A	Main	26	16	19	110
ECG	Bilddiagnostik	A	Main	6	10	10	94
General veterinary consultation / examination	Undersökning	B	Broad	62	37	147	62,841
Clinic visit base fee (poliklinik/grundavgift)	Undersökning	B	Broad	5	15	171	29,256
Sedation, weight not stated	Narkos/sedering	B	Broad	61	37	132	18,646
Re-visit consultation	Återbesök	B	Broad	12	29	137	12,974
Anaesthesia, type/duration not specified	Narkos/sedering	B	Broad	56	29	88	10,262
Abdominal ultrasound	Bilddiagnostik	B	Broad	54	23	63	5,696
Day-care / day-patient fee	Undersökning	B	Broad	58	23	64	5,580
Nurse fee, content not specified	Undersökning	B	Broad	55	35	44	4,430
Intravenous infusion	Behandling	B	Broad	38	11	91	4,249
X-ray base / equipment fee	Bilddiagnostik	B	Broad	10	31	34	4,132
Cytology, site and extent not stated	Provtagning/lab	B	Broad	39	33	89	4,095
Local anaesthesia	Narkos/sedering	B	Broad	59	34	118	3,928
Cardiac ultrasound	Bilddiagnostik	B	Broad	31	23	50	3,366
Histopathology, number of sites not stated	Provtagning/lab	B	Broad	35	32	41	2,267
Ultrasound, single organ	Bilddiagnostik	B	Broad	47	6	64	2,243
Dermatology consultation	Undersökning	B	Broad	26	17	13	2,184
Skin tumour removal, size not stated	Undersökning	B	Broad	58	33	18	2,141
Infusion, route not specified	Behandling	B	Broad	5	30	26	1,878
Separate cremation, weight not stated	Avlivning	B	Broad	57	32	55	1,645
Nurse consultation/visit	Undersökning	B	Broad	53	22	18	1,642
Wound treatment, level not stated	Undersökning	B	Broad	59	32	23	1,372
Sampling fee, cytology	Provtagning/lab	B	Broad	38	28	16	1,371
Bandage, size not specified	Behandling	B	Broad	51	30	23	1,331
Histopathology, 1-3 sites	Provtagning/lab	B	Broad	27	8	28	1,227
Pre-op profile, not specified	Provtagning/lab	B	Broad	19	8	21	1,204
Claw (nail bed) excision	Operationskostnader	B	Broad	55	32	59	1,200
Pancreas-specific lipase, method not stated	Provtagning/lab	B	Broad	10	24	36	1,142
Ear cleaning	Undersökning	B	Broad	57	31	40	1,127
Bacterial culture, resistance not stated	Provtagning/lab	B	Broad	35	12	31	1,038
Physiotherapy/rehab session	Rehabilitering	B	Broad	15	14	8	1,029
'Normalblod' profile, tier not stated	Blodprov	B	Broad	11	5	10	1,014
Cremation, type and weight not stated	Avlivning	B	Broad	56	7	42	957
Skin or ear cytology	Provtagning/lab	B	Broad	7	17	11	884
Subcutaneous infusion, administration	Behandling	B	Broad	53	12	62	870
Senior profile, clinic-defined	Provtagning/lab	B	Broad	53	6	9	867
Faecal parasite panel	Provtagning/lab	B	Broad	31	21	13	824
Professional dental cleaning (PTR)	Tandbehandling	B	Broad	6	28	44	807
Mammary tumour surgery, extent not stated	Operationskostnader	B	Broad	38	27	12	725

Liver profile, clinic-defined	Provtagning/lab	B	Broad	23	8	12	705
Urinalysis, extent not stated	Provtagning/lab	B	Broad	9	15	18	674
Sampling by fine-needle aspiration	Provtagning/lab	B	Broad	45	12	50	663
CT reading, areas not stated	Bilddiagnostik	B	Broad	8	5	5	648
Kidney profile, clinic-defined	Provtagning/lab	B	Broad	34	8	13	611
Oral cavity/throat inspection	Undersökning	B	Broad	43	23	37	579
Epidural/spinal analgesia	Narkos/sedering	B	Broad	12	11	5	573
Bandage, small/simple	Behandling	B	Broad	28	10	21	543
Pyometra surgery	Operationskostnader	B	Broad	45	25	30	509
Mammary tumour, solitary	Operationskostnader	B	Broad	45	5	25	403
FAST ultrasound	Bilddiagnostik	B	Broad	16	21	14	383
TPLO	Operationskostnader	B	Broad	7	8	6	356
Thyroid profile, content not stated	Blodprov	B	Broad	6	17	5	329
Rehab card, 5 sessions	Rehabilitering	B	Broad	10	6	5	311
Cystotomy / urolith removal	Operationskostnader	B	Broad	37	21	13	309
Cast application/change/removal	Operationskostnader	B	Broad	8	5	5	228
Eyelid tumour surgery	Operationskostnader	B	Broad	22	25	9	205
Oxygen therapy	Narkos/sedering	B	Broad	17	11	8	201
X-ray image reading	Bilddiagnostik	B	Broad	23	11	5	193
Castration, male dog	Operationskostnader	B	Broad	25	12	24	158
Toe amputation, dog	Operationskostnader	B	Broad	24	17	19	143
Spay (ovariohysterectomy), bitch	Operationskostnader	B	Broad	22	9	8	136
Tail amputation, dog	Operationskostnader	B	Broad	26	18	7	114
Splenectomy	Operationskostnader	B	Broad	15	14	9	107
Coat care	Förebyggande beh	B	Broad	33	8	10	86
Joint lavage	Behandling	B	Broad	11	5	9	66

Notes: Measure: the services of the Broad basket: the smallest basket that contains each service, its grade and item category (none for the Core services), the number of Evidensia, AniCura and never-acquired independent clinics (without Distriktsveterinärerna) that bill it between January 2019 and March 2025 (the months in which the data cover the most insurers), and its invoice lines at all clinics in the same period. *Sample:* a service of grade A or B enters a basket only if at least 5 clinics of each of these three groups bill it, not counting the 12 acquired clinics. *Definitions:* grades, Section B.4 (grade A: the content of the service is fixed by its definition; grade B: the content may vary between clinics); baskets, Section B.5 (the Main basket consists of the Core services and the services of grade A, and the Broad basket adds the services of grade B).

C Price comparisons: robustness and more results

This section gives the chain premium under one change at a time (Table C.1) and under every price definition (Figure C.1), what chains bill in a visit (Table C.2), the premium of the acquired clinics before and after acquisition and of other chain clinics (Table C.3), and the premium by the owner's local concentration (Table C.4).

Table C.1: Robustness of the chain premium, one change at a time

	Basket price index			Line price, category FE		Line price	Spending
Specification	Core	Main	Broad	BERT	LLM	free-text FE	per visit
Baseline	0.259***	0.259***	0.313***	0.286***	0.295***	0.232***	0.137***
	(0.018)	(0.018)	(0.018)	(0.049)	(0.050)	(0.029)	(0.031)
Evidensia (vs all other clinics)	0.285***	0.296***	0.353***	0.387***	0.400***	0.279***	0.136***
	(0.019)	(0.018)	(0.017)	(0.044)	(0.045)	(0.028)	(0.033)
AniCura (vs all other clinics)	0.179***	0.136***	0.200***	0.004	0.006	0.136***	0.137***
	(0.025)	(0.021)	(0.020)	(0.071)	(0.070)	(0.039)	(0.049)
No common ownership in the city	0.254***	0.259***	0.314***	0.210***	0.214***	0.224***	0.129***
	(0.022)	(0.022)	(0.020)	(0.062)	(0.062)	(0.032)	(0.035)
Without single-clinic cities	0.255***	0.255***	0.307***	0.274***	0.284***	0.230***	0.135***
	(0.019)	(0.019)	(0.018)	(0.050)	(0.051)	(0.029)	(0.031)
Without Distriktsveterinärerna	0.267***	0.257***	0.313***	0.290***	0.300***	0.234***	0.136***
	(0.018)	(0.019)	(0.018)	(0.050)	(0.051)	(0.029)	(0.031)
Without hospitals	0.261***	0.261***	0.318***	0.298***	0.299***	0.237***	0.176***
	(0.020)	(0.020)	(0.019)	(0.047)	(0.047)	(0.021)	(0.033)
Without clinic group B	0.263***	0.264***	0.318***				
	(0.018)	(0.019)	(0.018)				
Clinic-years weighted equally	0.259***	0.251***	0.307***	0.241***	0.246***	0.217***	0.078***
	(0.018)	(0.018)	(0.017)	(0.035)	(0.035)	(0.021)	(0.025)
Weighted by invoice lines	0.301***	0.261***	0.316***				
	(0.019)	(0.024)	(0.024)				
Service-level regression	0.246***	0.249***	0.304***				
	(0.018)	(0.019)	(0.019)				
Basket ratio	0.224***	0.205***	0.248***				
	(0.020)	(0.019)	(0.020)				
Looser screen (either chain)		0.274***	0.312***				
		(0.018)	(0.018)				
No screen		0.276***	0.314***				
		(0.018)	(0.018)				
Only fixed-quantity services		0.259***	0.294***				
		(0.018)	(0.017)				
Without the prescription fee	0.268***	0.260***	0.315***				
	(0.020)	(0.019)	(0.018)				
Core texts incl. 24 contested texts	0.258***	0.259***	0.313***				
	(0.018)	(0.018)	(0.018)				
Core texts: written rule	0.274***	0.262***	0.315***				
	(0.017)	(0.018)	(0.018)				
No minimum number of services	0.243***	0.257***	0.311***				
	(0.017)	(0.018)	(0.018)				
$\geqslant$ 100 visits per clinic	0.263***	0.261***	0.314***	0.285***	0.294***	0.229***	0.137***
	(0.018)	(0.018)	(0.018)	(0.049)	(0.051)	(0.028)	(0.031)
All clinics, no minimums	0.239***	0.259***	0.302***	0.277***	0.288***	0.224***	0.127***
	(0.016)	(0.017)	(0.016)	(0.046)	(0.047)	(0.027)	(0.029)
All visits, incl. visits reported without invoice lines							0.145***
							(0.031)
Insurer FE				0.288***	0.298***	0.233***	0.137***
				(0.050)	(0.051)	(0.029)	(0.031)
City FE	0.242***	0.223***	0.272***	0.239***	0.245***	0.207***	0.067**
	(0.021)	(0.023)	(0.022)	(0.049)	(0.052)	(0.025)	(0.031)

Notes: Measure: the chain premium (the coefficient on the chain dummy) under one change at a time. Columns 1–3: the basket price indices of Table 1, Panel A; columns 4–6: the log price of an invoice line, with item-category fixed effects (BERT or LLM categories) or invoice-text fixed effects; column 7: log spending per visit as in Table 1, Panel B. *Sample:* in the baseline, 1,393 (Core), 1,637 (Main) and 1,846 (Broad) clinic-years, 2,774,247 invoice lines (category fixed effects; slightly fewer with invoice-text fixed effects) and 335,924 visits. *Specification:* the preferred specification of each measure: for the indices, one observation per clinic and calendar year, weighted by the number of services priced; for line prices, fixed effects for year, hospital, region, city type, diagnosis type, breed type, revisit and emergency. Each row changes one thing. Evidensia and AniCura: separate dummies for the two chains in one regression. No common ownership: observations whose owner has no other clinic in the city at any time in the data. Without Distriktsveterinärerna (the state-run district veterinary service), single-clinic cities, hospitals or clinic group B (Table B.4): those clinics are dropped (for lines and visits: the visits at hospitals). Clinic-years weighted equally: no weights for the indices; for lines and visits, weights of one over the number of observations of the clinic-year. Weighted by invoice lines: by the invoice lines behind the clinic-year's prices. Service-level regression: one observation per clinic, service and calendar year, with service-by-year fixed effects. Basket ratio: the log of the sum of the clinic's prices over the sum of the never-acquired clinics' mean prices of the same services. Screen, fixed-quantity, prescription-fee and Core-text rows: the basket is rebuilt with that choice; the looser screen requires at least 5 independent clinics and 5 clinics of either chain (instead of each chain). No minimum number of services: clinic-years with fewer than 3

services are kept. At least 100 visits per clinic: instead of 10. All clinics: no clinic restrictions (visits under Svenska Veterinärgruppen ownership are still excluded) and, for the indices, no minimum number of reference clinics or services. All visits: every visit with positive spending, including the 60,795 visits one insurer reported without invoice lines (almost all in 2025), whose emergency status is not observed. Insurer fixed effects: line prices and spending only (the price index pools insurers). City FE: municipality fixed effects added to the preferred specification. Empty cells: the change does not apply. *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

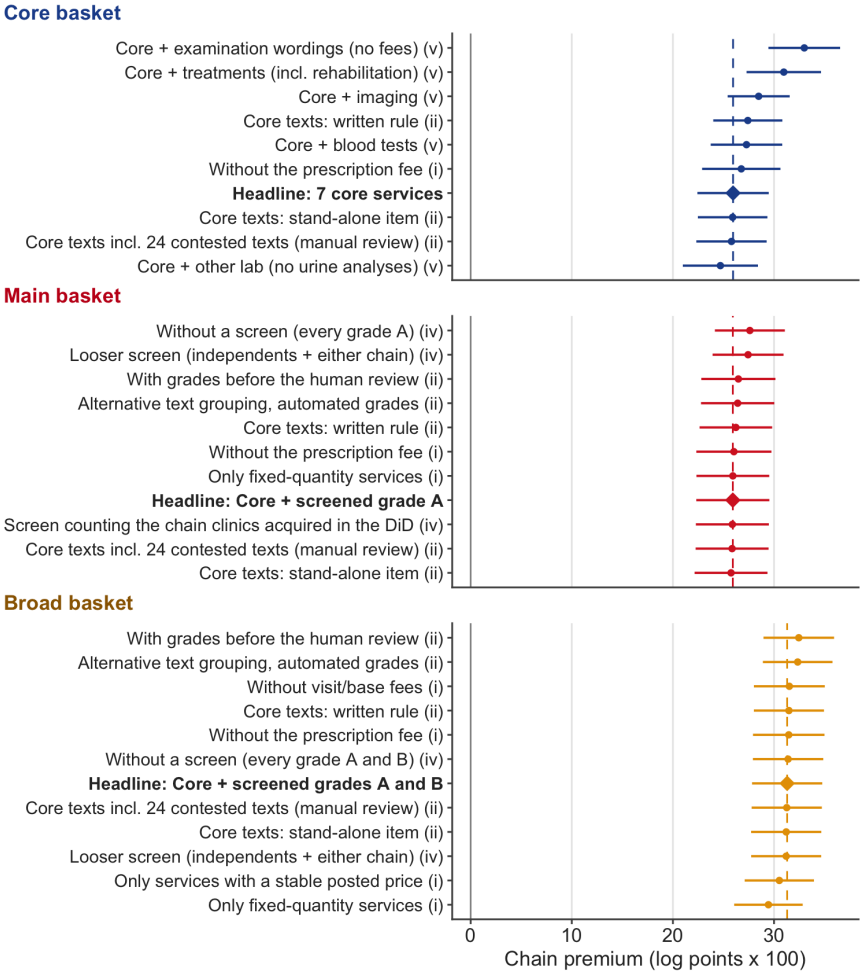


Figure C.1: Chain premium for every price definition

Notes: Measure: the chain premium (the coefficient on the chain dummy) for each definition of the basket price index, sorted by the estimate within each basket; the diamond and the dashed line mark the definition used in the paper. The numeral after each label gives the kind of change: (i) which services are in the basket; (ii) how invoice texts are assigned to services; (iii) how a clinic's price of a service in a year is measured; (iv) the screen that selects the services; (v) the Core basket plus the services of one domain; (vi) how the prices are aggregated. Stable posted price: on average over clinic-months, at least 80% of a clinic's lines of the service are at its modal price of the month. Looser screen: the service is billed by at least 5 independent clinics and at least 5 clinics of either chain (in the paper: of each chain). *Specification:* the preferred specification of Table 1; changes of kind (iii) and (vi) are estimated for the effect of acquisition only (Figure E.1), and the basket ratio is a row of Table C.1. *Definitions:* Section B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

Table C.2: Probability of including lab tests, imaging, ultrasound and sedation in a visit

Outcome (0/1)	(1)	(2)	(3)	(4)	(5)	**(6)**	Mean of outcome
Blood or lab test	0.037***	0.017	0.010	0.007	-0.010	-0.011	0.531
	(0.013)	(0.014)	(0.013)	(0.012)	(0.011)	(0.010)	
Imaging	0.044	-0.021	-0.034**	-0.035***	-0.033***	-0.035***	0.345
	(0.028)	(0.015)	(0.014)	(0.013)	(0.011)	(0.010)	
Ultrasound	0.059**	0.004	-0.007	0.000	0.002	0.000	0.166
	(0.028)	(0.019)	(0.015)	(0.012)	(0.011)	(0.011)	
Sedation	0.033*	0.020	0.029**	0.021	0.003	0.002	0.333
	(0.017)	(0.015)	(0.014)	(0.014)	(0.013)	(0.013)	
Year	✓	✓	✓	✓	✓	✓	
Hospital		✓	✓	✓	✓	✓	
Region			✓	✓	✓	✓	
City type				✓	✓	✓	
Diagnosis type					✓	✓	
Breed type, revisit, emergency, pet age						✓	
N (visits)	337,099	337,099	337,099	337,099	336,209	336,206	

Notes: Measure: the probability that the visit includes the service; mean of outcome: the share of visits with the service in the sample of column (6). *Sample:* one observation per visit of the cross-sectional sample with invoice lines and positive spending (the visits reported without invoice lines have no outcome); the number of observations differs across columns because observations with a missing control, or alone in a fixed-effect cell, are dropped. *Specification:* linear probability models on a chain dummy (AniCura or Evidensia at the visit; reference: independent clinics), with the fixed effects marked below each column; the preferred specification is in bold. *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

Table C.3: Premium of the acquired clinics before and after acquisition, and of other chain clinics

Measure	Acquired clinics, before	Acquired clinics, after	Acquired clinics, 3+ calendar years after	Other chain clinics	N	Unit
Core	-0.060	0.109***	0.206***	0.269***	1,369	clinic-years
	(0.062)	(0.028)	(0.034)	(0.020)		
Main	-0.029	0.116***	0.209***	0.271***	1,611	clinic-years
	(0.055)	(0.025)	(0.028)	(0.020)		
Broad	-0.039	0.163***	0.244***	0.325***	1,819	clinic-years
	(0.052)	(0.028)	(0.027)	(0.019)		
Line price, category FE (BERT categories)	-0.424***	0.222***	0.261***	0.230***	2,750,504	invoice lines
	(0.090)	(0.074)	(0.071)	(0.055)		
Line price, category FE (LLM categories)	-0.458***	0.233***	0.278***	0.234***	2,750,504	invoice lines
	(0.089)	(0.075)	(0.070)	(0.056)		
Line price, free-text FE	0.056	0.161***	0.188***	0.254***	2,748,942	invoice lines
	(0.047)	(0.043)	(0.047)	(0.031)		
Spending per visit	0.081	-0.001	-0.003	0.164***	331,783	visits
	(0.059)	(0.046)	(0.055)	(0.033)		

Notes: Measure: the premium, relative to the never-acquired clinics, of the 12 acquired clinics before their acquisition (column 1) and after it (column 2), and of the observations under AniCura or Evidensia ownership of the clinics not acquired during the data (column 4); column 3 gives the acquired clinics observed from the third calendar year after the year of the closing on (for the Main basket, line prices and spending per visit, 11 clinics observed in 2025–2026; for clinics acquired late in a year, the third calendar year begins about two years after the closing). *Sample:* for each measure, the sample of its preferred specification (Table 1 and Table C.1) without the clinics acquired during the data other than the 12; spending per visit uses the visits with at least one priced invoice line; N is in the unit of the last column. *Specification:* columns 1, 2 and 4, one regression per row on dummies for the three groups, with the controls and weights of the preferred specification of the measure; column 3, a second regression in which the years after the acquisition are split into earlier and later years, of which only the later are reported; category fixed effects use the experts' categories, extended to the other invoice texts by BERT or by LLM agents. *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

Table C.4: Market concentration: three measures of the owner's local concentration

Panel A: owner's share of the city's clinics	Basket price indices			Line prices			
	Core	Main	Broad	Category FE (BERT)	Category FE (LLM)	Free-text FE	Spending per visit
Chain	0.262***	0.265***	0.318***	0.320***	0.329***	0.252***	0.141***
	(0.018)	(0.018)	(0.018)	(0.054)	(0.056)	(0.031)	(0.033)
Owner's share of the city's clinics	-0.067	-0.119	-0.111	-0.440**	-0.433**	-0.231**	0.002
	(0.072)	(0.076)	(0.079)	(0.183)	(0.183)	(0.101)	(0.092)
Chain × share of clinics	0.091	0.176*	0.185*	0.424*	0.426*	0.210*	0.243*
	(0.083)	(0.100)	(0.103)	(0.241)	(0.244)	(0.122)	(0.139)
Panel B: owner's share of the city's visits							
	Core	Main	Broad	Category FE (BERT)	Category FE (LLM)	Free-text FE	Spending per visit
Chain	0.251***	0.258***	0.308***	0.285***	0.294***	0.238***	0.143***
	(0.019)	(0.020)	(0.019)	(0.067)	(0.071)	(0.034)	(0.034)
Owner's share of the city's visits	0.038	0.008	0.021	-0.004	0.002	0.002	0.030
	(0.041)	(0.042)	(0.043)	(0.146)	(0.157)	(0.046)	(0.060)
Chain × share of visits	0.006	0.079	0.072	-0.012	-0.017	0.053	0.150*
	(0.054)	(0.054)	(0.062)	(0.202)	(0.212)	(0.061)	(0.086)
Panel C: number of the chain's clinics in the city							
	Core	Main	Broad	Category FE (BERT)	Category FE (LLM)	Free-text FE	Spending per visit
Chain	0.261***	0.261***	0.315***	0.309***	0.320***	0.231***	0.143***
	(0.018)	(0.018)	(0.018)	(0.051)	(0.054)	(0.031)	(0.035)
Chain × number of clinics	0.010	0.007	0.009	0.036	0.037	-0.002	0.012
	(0.008)	(0.007)	(0.008)	(0.022)	(0.023)	(0.008)	(0.018)
N	1,386	1,630	1,839	2,771,248	2,771,248	2,769,522	335,638
Cities	135	153	157	159	159	159	159

Notes: Measure: the chain premium and its interaction with the owner's local concentration in the city (the municipality): Panel A, the owner's share of the city's clinics; Panel B, its share of the city's visits in the calendar year; Panel C, the number of the chain's clinics in the city, which the data count for the chains only, so that Panel C has the interaction but no main effect. *Sample:* each column uses the same observations in all three panels: those with a city and all three measures (for basket price indices, clinic-years in which the share of the city's clinics is observed on at least one visit). N counts clinic-years (basket price indices, weighted by the number of services priced), invoice lines (line prices) or visits (spending per visit, as in Table 1, Panel B). *Specification:* one regression per column and panel: the preferred specification of the measure (Table 1 and Table C.1) with the chain dummy, the concentration measure and their interaction; each measure is centred at its mean among the chain observations (for the indices, weighted by the number of services priced; Main basket: 0.21, 0.68 and 2.0), so Chain is the premium at a typical chain level, and the interaction does not depend on the centring; category fixed effects use the experts' categories, extended to the other invoice texts by BERT or by LLM agents. *Definitions:* Section B.2. Standard errors clustered by city in parentheses, since the measures of concentration vary across cities. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

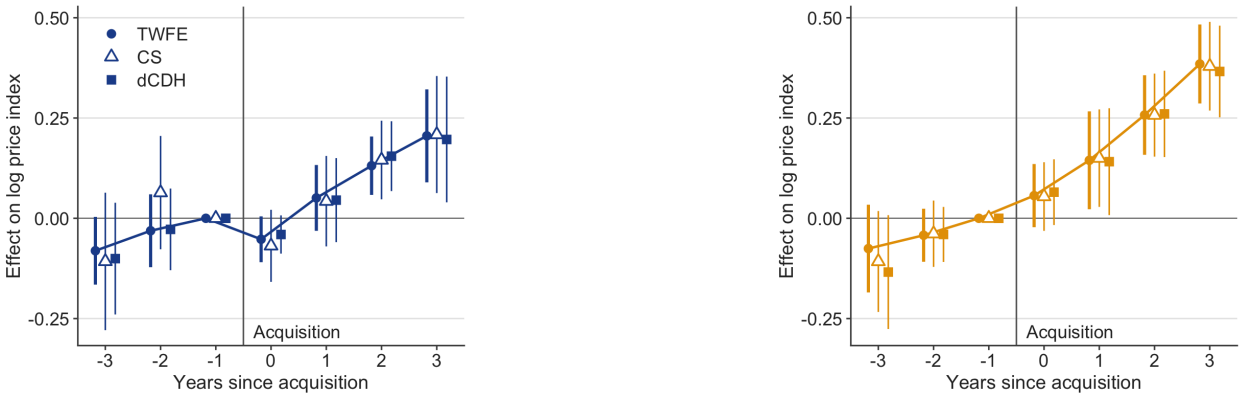
D Effect of acquisition: robustness

This section gives the effect of acquisition under the three estimators (Table D.1, Figure D.1), under other estimator choices (Table D.2), with wild-cluster bootstrap p-values (Table D.3), under other samples and specifications (Table D.4), leaving out one clinic or clinic group at a time (Figure D.2), and the event study under each alternative of Table D.4 (Figure D.3).

Table D.1: Difference-in-differences estimates of the effect of acquisition by a chain

	TWFE		CS, never treated		dCDH, never switchers		Clinics		
	Estimate	p-trend	Estimate	p-trend	Estimate	p-placebo	Treated	Controls	N (clinic-years)
Panel A: basket price indices									
Core	0.079**	0.154	0.082	0.177	0.081*	0.353	12	146	685
	(0.035)		(0.052)		(0.043)				
Main	0.142***	0.955	0.157***	0.953	0.150***	0.802	12	177	894
	(0.039)		(0.043)		(0.045)				
Broad	0.199***	0.321	0.210***	0.207	0.198***	0.165	12	186	1,058
	(0.043)		(0.045)		(0.046)				
Panel B: spending per visit									
Spending per visit	0.061	0.331	0.099**	0.377	0.078	0.201	12	188	1,176
	(0.051)		(0.049)		(0.052)				
Panel C: price versus intensity (visits with a priced invoice line)									
Log spending per visit	0.055	0.329	0.092*	0.392	0.072	0.202	12	188	1,172
	(0.052)		(0.050)		(0.052)				
Log spending per visit at fixed category prices	-0.142**	<0.001	-0.135**	<0.001	-0.128*	<0.001	12	188	1,172
	(0.069)		(0.067)		(0.072)				
Log number of priced invoice lines	-0.062	<0.001	-0.063	0.007	-0.049	<0.001	12	188	1,172
	(0.067)		(0.066)		(0.069)				

Notes: Measure: the effect of acquisition on the basket price indices (Panel A), on log spending per visit (Panel B; the clinic-year mean of the log total billed per visit, which also reflects the number and kind of services) and on three outcomes per visit that separate prices from the services provided (Panel C; fixed category prices as in Table 1). Estimate: the mean effect over years 0 to 3; p-trend and p-placebo: joint test of years -3 and -2. *Sample:* main DiD sample, one observation per clinic and year (October to September; year 0 contains the closing date), 2017/18–2025/26; Panel C uses the visits of this sample with at least one priced invoice line (the visit rule of Table 1, Panel B), so its first row differs slightly from Panel B, which also includes the visits reported without invoice lines. *Specification:* TWFE, two-way fixed effects; CS, Callaway and Sant'Anna (2021); dCDH, de Chaisemartin and D'Haultfœuille (2020); in CS (never treated) and dCDH (never switchers) the never-acquired clinics of the main DiD sample are the controls. *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.



(a) Core basket

(b) Broad basket

Figure D.1: Effect of acquisition on the prices of the Core and the Broad basket: TWFE, CS and dCDH

Notes: Measure: (a) the Core and (b) the Broad basket price index, by year since acquisition, relative to year -1. *Specification:* TWFE, CS and dCDH, with data and specification as in Figure 1. *Estimates:* Table D.1. *Definitions:* Section B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

Table D.2: Estimator choices, the three baskets

Estimator	Core	Main	Broad
TWFE, years weighted by acquired clinics (baseline)	0.079**	0.142***	0.199***
	(0.035)	(0.039)	(0.043)
TWFE, years weighted equally	0.084**	0.151***	0.211***
	(0.036)	(0.039)	(0.043)
TWFE, end points binned at ± 3 (every clinic-year)	0.093**	0.147***	0.206***
	(0.036)	(0.039)	(0.044)
TWFE with a common linear trend for acquired clinics	-0.019	0.137**	0.109
	(0.079)	(0.067)	(0.103)
CS, never treated (baseline)	0.082	0.157***	0.210***
	(0.052)	(0.043)	(0.045)
CS, not yet treated	0.080	0.155***	0.209***
	(0.053)	(0.042)	(0.044)
dCDH, never switchers (baseline)	0.081*	0.150***	0.198***
	(0.043)	(0.045)	(0.046)
dCDH, all switchers	0.082*	0.149***	0.197***
	(0.042)	(0.045)	(0.047)

Notes: Measure: the mean effect of acquisition over years 0 to 3 under alternative choices within each estimator. *Specification:* as in Table D.1, changing one choice at a time. TWFE: the years since acquisition weighted by the number of acquired clinics observed in each (baseline) or equally; end points binned, which keeps every clinic-year by including the years before -3 and after $+3$ in the indicators for -3 and $+3$ (the baseline drops them); or a common linear trend, which replaces the pre-acquisition indicators with one linear trend in years since acquisition, common to the acquired clinics. CS and dCDH: clinics acquired later added to the controls (CS, not yet treated; dCDH, all switchers). *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

Table D.3: Wild-cluster bootstrap p-values (TWFE), the three baskets

Basket	TWFE, mean of years 0–3	SE	p (normal approximation)	p (wild-cluster bootstrap)	Acquired clinics	N (clinic-years)
Core	0.079	0.035	0.025	0.051	12	685
Main	0.142	0.039	<0.001	0.005	12	894
Broad	0.199	0.043	<0.001	<0.001	12	1,058

Notes: Measure: p-values for the TWFE mean effect over years 0 to 3 (null hypothesis: no effect), from the normal approximation (standard error clustered by clinic) and from a wild-cluster restricted bootstrap with Webb weights, 999 draws and clinics as clusters. *Specification:* as in Table D.1. *Definitions:* Section B.2.

Table D.4: Robustness of the effect of acquisition, Main basket

	TWFE		CS, never treated		dCDH, never switchers		Clinics		
	Estimate	p-trend	Estimate	p-trend	Estimate	p-placebo	Treated	Controls	N (clinic-years)
Baseline	0.142***	0.955	0.157***	0.953	0.150***	0.802	12	177	894
	(0.039)		(0.043)		(0.045)				
Hospitals excluded	0.161***	0.216	0.172***	0.095	0.171***	0.546	10	169	829
	(0.043)		(0.048)		(0.050)				
Without Distriktsveterinärerna	0.140***	0.934	0.154***	0.948	0.147***	0.823	12	147	827
	(0.040)		(0.043)		(0.046)				
Controls: no clinics in acquisition cities	0.142***	0.144	0.157***	0.218	0.150***	0.398	12	158	802
	(0.040)		(0.042)		(0.046)				
Without the presence and 10-visit rules	0.137***	0.447	0.155***	0.632	0.148***	0.796	12	293	1,133
	(0.039)		(0.044)		(0.044)				
$\geqslant$ 100 visits per clinic (baseline $\geqslant$ 10)	0.143***	0.937	0.155***	0.888	0.151***	0.802	12	162	855
	(0.039)		(0.041)		(0.044)				
Without the AniCura acquisition	0.159***	0.971	0.168***	0.823	0.167***	0.938	11	177	888
	(0.040)		(0.044)		(0.046)				
Without clinic group A (5 clinics)	0.214***	0.366	0.256***	0.967	0.222***	0.095	7	177	862
	(0.037)		(0.038)		(0.030)				
Without clinic group B (4 clinics)	0.060**	0.068	0.074**	0.096	0.053	0.019	8	177	869
	(0.031)		(0.029)		(0.034)				
Looser screen: $\geqslant$ 5 independents and $\geqslant$ 5 clinics of either chain	0.148***	0.516	0.166***	0.587	0.157***	0.584	12	179	915
	(0.042)		(0.044)		(0.049)				
No screen	0.160***	0.441	0.174***	0.509	0.167***	0.403	12	182	963
	(0.042)		(0.043)		(0.049)				
Core texts incl. 24 contested texts (manual review)	0.146***	0.892	0.161***	0.891	0.154***	0.820	12	177	895
	(0.038)		(0.041)		(0.044)				
Core texts: written rule	0.148***	0.818	0.166***	0.822	0.156***	0.530	12	176	885
	(0.040)		(0.042)		(0.047)				
Without the prescription fee	0.128***	0.716	0.142***	0.773	0.136***	0.329	12	176	862
	(0.042)		(0.050)		(0.050)				
Only fixed-quantity services	0.147***	0.718	0.159***	0.937	0.153***	0.576	12	177	893
	(0.039)		(0.042)		(0.045)				
Basket ratio: log(sum of prices / sum of never-acquired clinics' mean prices)	0.104**	0.062	0.117**	0.223	0.101*	0.057	12	177	894
	(0.046)		(0.051)		(0.055)				
No minimum number of services	0.161***	0.477	0.170***	0.560	0.168***	0.440	12	187	1,084
	(0.042)		(0.043)		(0.045)				
Weighted by services priced (no minimum number of services)	0.112***	0.259					12	187	1,084
	(0.036)								

Notes: Measure: the effect of acquisition on the Main basket price index under one change at a time from the baseline of Table D.1; columns as in Table D.1. *Specification:* as in Table D.1, changing one thing at a time. Without Distriktsveterinärerna: these never-acquired clinics are dropped. No clinics in acquisition cities: never-acquired clinics in the city of one of the 12 acquired clinics are dropped (possible competitors, Table F.3). Without the presence and 10-visit rules: clinics without a visit in each of 2022–2025 or with fewer than 10 visits are kept. Without clinic group A or B: the 5 or 4 clinics of one acquisition (Table B.4). Screen, Core-text, prescription-fee and fixed-quantity rows: the basket is rebuilt with that choice (fixed quantity: the screened grade A services without anaesthesia and sedation, surgery and operating costs, dental care, rehabilitation and infusions, and without services whose label leaves the weight, size, extent or content unstated; the Core services are always kept). Basket ratio: as in Table C.1. Weighted by services priced: TWFE weighted by the number of services the clinic prices in the year, without a minimum number of services (TWFE only). *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

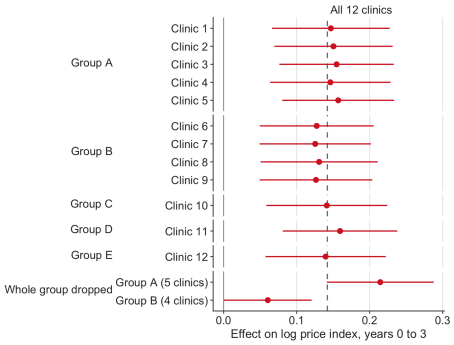


Figure D.2: Effect of acquisition on the Main basket, leaving out one acquired clinic or one clinic group at a time

Notes: Measure: the TWFE mean effect of acquisition on the Main basket price index over years 0 to 3, leaving out one acquired clinic at a time or, in the panel "Whole group dropped", every clinic of clinic group A or B at once (Table D.4); dashed line: the estimate with all 12 acquired clinics (Table D.1). *Sample:* clinics numbered by closing date and grouped by clinic group (Table B.4). *Specification:* as in Figure 1. *Definitions:* Section B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

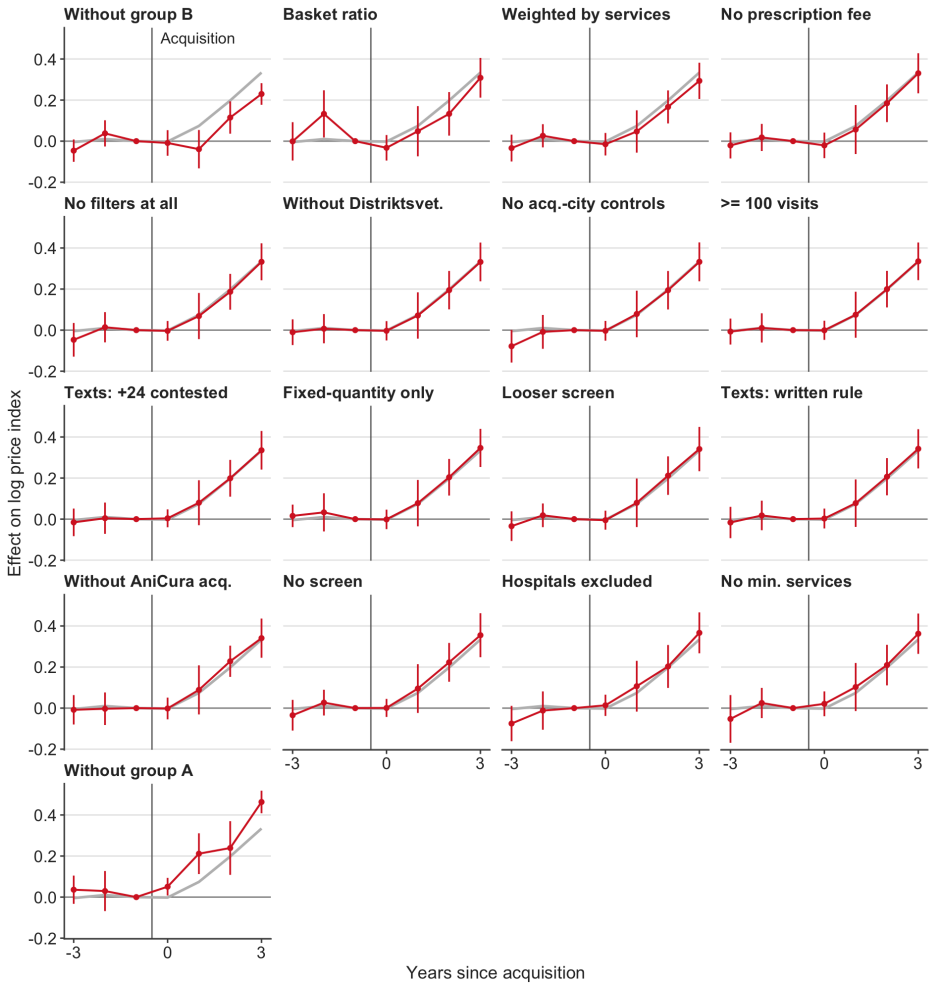


Figure D.3: Effect of acquisition on the Main basket under each alternative of Table D.4 (TWFE)

Notes: Measure: TWFE event-study estimates of the effect of acquisition on the Main basket price index under each alternative of Table D.4 (red), with the baseline in grey; panels are sorted by the mean effect over years 0 to 3. *Specification:* as in Figure 1, panel (b), with the alternatives described in the notes to Table D.4. *Estimates:* Table D.4. *Definitions:* Section B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

E Price measures

This section gives the effect of acquisition under every price definition (Figure E.1), on indices of all invoice lines and on spending per visit (Table E.1, Figure E.2), and for the same services within a clinic (Table E.2).

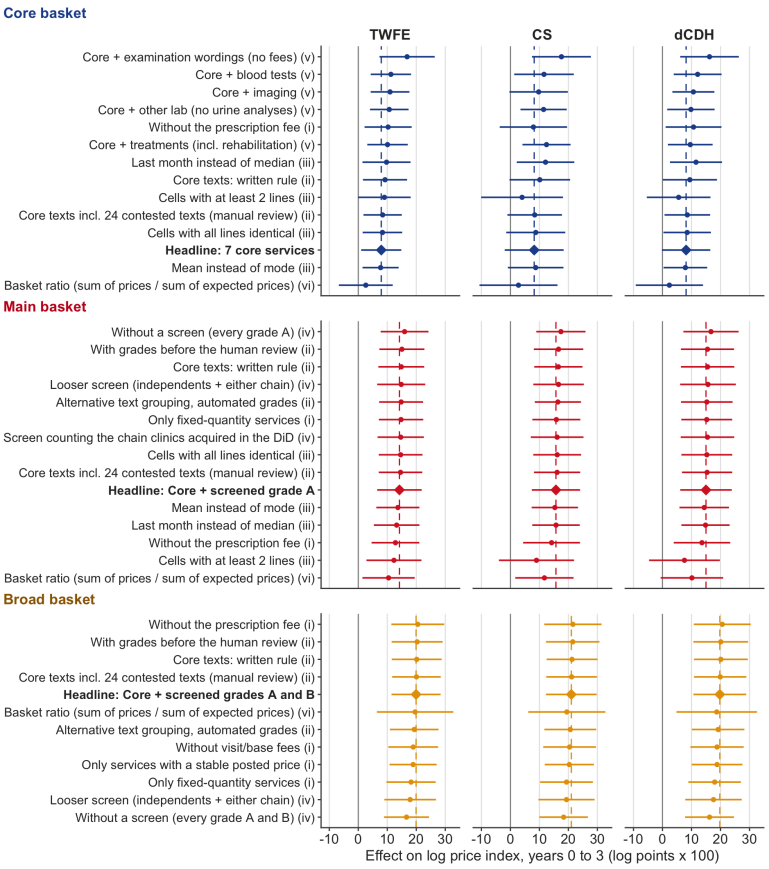


Figure E.1: Effect of acquisition for every price definition: TWFE, CS and dCDH

Notes: Measure: the mean effect of acquisition over years 0 to 3 for each definition of the basket price index, sorted by the TWFE estimate within each basket; the diamond and the dashed line mark the definition used in the paper; labels and kinds of change (i)–(vi) as in Figure C.1, which gives the chain premium of the definitions of kinds (i), (ii), (iv) and (v). The definitions with the stand-alone Core texts are not shown (each is within 0.01 log points of the baseline definition). *Specification:* TWFE, CS (never treated) and dCDH (never switchers), with the never-acquired clinics as controls (in TWFE, also acquired clinics in other years); specification and sample as in Table D.1. *Definitions:* Section B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

Table E.1: Effect of acquisition on indices of all invoice lines: within category and within free text

	TWFE		CS, never treated		dCDH, never switchers		Clinics		
	Estimate	p-trend	Estimate	p-trend	Estimate	p-placebo	Treated	Controls	N (clinic-years)
Category index (BERT categories)	0.210***	0.109	0.238***	0.256	0.207***	0.032	12	188	1,165
	(0.051)		(0.056)		(0.055)				
Category index (LLM categories)	0.198***	0.130	0.237***	0.756	0.205***	0.022	12	188	1,165
	(0.057)		(0.059)		(0.059)				
Free-text index	0.094***	0.043	0.114***	0.026	0.094***	0.107	12	188	1,165
	(0.034)		(0.035)		(0.033)				

Notes: Measure: the effect of acquisition on two price indices built from all invoice lines: the free-text index, the clinic's mean log line price of an invoice text (by emergency status) relative to the never-acquired clinics of the main DiD sample, averaged over its invoice texts with equal weights; and the category index, the same for the sum per visit of the lines of an item category, relative to the never-acquired clinics for the same category, diagnosis type and emergency status, with the experts' categories extended to the other invoice texts by BERT or by the LLM agents. *Sample:* one observation per clinic and year, as in Table D.1. *Specification:* columns and estimators as in Table D.1. *Definitions:* Section B.2; categories, Sections B.3 and B.4. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

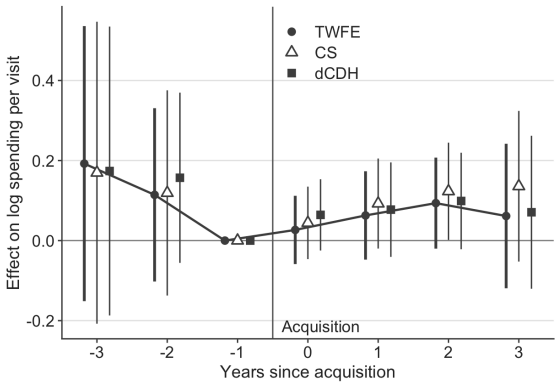


Figure E.2: Effect of acquisition on spending per visit: TWFE, CS and dCDH

Notes: Measure: log spending per visit (the clinic-year mean of the log total billed per visit, excluding VAT), by year since acquisition, relative to year -1; spending per visit is not a price: it also reflects the number and kind of services. *Specification:* TWFE, CS and dCDH, with data and specification as in Figure 1. *Estimates:* Table D.1, Panel B. *Definitions:* Section B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

The same services within a clinic. Since the index measures every service against its own reference price, it does not rise because a clinic starts to bill more expensive services. The set of services that a clinic bills can, however, change at an acquisition, for example when the acquirer's invoice system bills in one line what used to be billed in several. Table E.2 removes this channel by following the price of each service within a clinic, so that only services billed both before and after the acquisition contribute. For the Main basket the effect is then 8% instead of 15%: prices rise service by service, and the rest of the effect comes from changes in the set of services billed.

Table E.2: Effect of acquisition: the same services within a clinic versus the index

	TWFE		CS, never treated		dCDH, never switchers		Clinics	
	Estimate	p-trend	Estimate	p-trend	Estimate	p-placebo	Treated	Controls
Core index (Table D.1)	0.079**	0.154	0.082	0.177	0.081*	0.353	12	146
	(0.035)		(0.052)		(0.043)			
Core same services (clinics weighted equally)	0.042	0.063	0.054	0.042	0.055	0.082	12	146
	(0.035)		(0.040)		(0.043)			
Main index (Table D.1)	0.142***	0.955	0.157***	0.953	0.150***	0.802	12	177
	(0.039)		(0.043)		(0.045)			
Main same services (clinics weighted equally)	0.077***	0.159	0.107**	0.065	0.083***	0.330	12	177
	(0.029)		(0.045)		(0.029)			
Broad index (Table D.1)	0.199***	0.321	0.210***	0.207	0.198***	0.165	12	186
	(0.043)		(0.045)		(0.046)			
Broad same services (clinics weighted equally)	0.075**	0.003	0.138***	0.008	0.072**	0.079	12	186
	(0.032)		(0.038)		(0.029)			

Notes: Measure: the effect of acquisition on basket prices, measured by the price index (index rows: the estimates of Table D.1) and by the same services within a clinic (same-services rows: the log price of the service relative to the never-acquired clinics of the main DiD sample, with clinic-by-service and year fixed effects, so that only changes in the price of a service that the clinic prices both before and after the acquisition identify the effect); the difference between the rows is due to changes over time in the set of services that a clinic prices. *Sample:* index rows, one observation per clinic and year; same-services rows, one per clinic, service and year, each clinic-service weighted by one over the number of services the clinic prices in the event window. *Specification:* columns and estimators as in Table D.1; the Clinics columns count clinics (in CS and dCDH the units are clinic-services). *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

F Mechanisms

This section gives the effect of acquisition without common ownership in the city (Table F.1), how each owner sets prices (Table F.2, Figure F.1), the prices of competing clinics (Table F.3) and the IT switch (Section F.1).

Table F.1: Effect of acquisition without common ownership in the city

	TWFE		CS, never treated		dCDH, never switchers		Clinics		
	Estimate	p-trend	Estimate	p-trend	Estimate	p-placebo	Treated	Controls	N (clinic-years)
Core	0.087**	0.250	0.093*	0.945	0.096	0.020	6	146	648
	(0.039)		(0.050)		(0.065)				
Main	0.109**	0.725	0.125**	0.881	0.113	0.011	6	177	854
	(0.054)		(0.057)		(0.087)				
Broad	0.188***	0.491	0.208***	0.079	0.185**	0.363	6	186	1,017
	(0.062)		(0.058)		(0.073)				
Spending per visit	0.048	0.581	0.094	0.636	0.070	0.686	6	188	1,135
	(0.072)		(0.079)		(0.082)				

Notes: Measure: the effect of acquisition for the acquisitions without common ownership in the city; columns and estimators as in Table D.1. *Sample:* a clinic is dropped if its owner, as named in the clinic register, has another clinic in the same city at any time in the data; independent owners are not named, so no never-acquired clinic is dropped, and the acquired clinics are those whose acquirer owns no other clinic in the city (among them 1 of the 4 clinics of clinic group B). *Specification:* as in Table D.1; the dCDH placebo test rejects at the 5% level for the Core basket (p = 0.020) and the Main basket (p = 0.011). *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

Table F.2: How prices are set, by owner

Fact	Independent	Evidensia	AniCura
Share of prices changing per month	0.082	0.143	0.110
Months a price lasts (1 / share)	12.179	6.989	9.064
Changes in January-February (2020-24)	0.275	0.189	0.096
Median size of a change (log points)	0.082	0.051	0.119
Changes that are increases	0.855	0.891	0.764
Prices that are a multiple of 10 kr	0.384	0.199	0.510
Same price as another clinic of the group	0.322	0.567	0.377
At the chain list price (independents: Evidensia)	0.024	0.489	0.478
Cross-clinic SD of the same service log price	0.350	0.177	0.215
Clinics	272	46	23

Notes: Measure: facts on the monthly modal prices of the Main-basket services (clinic-months with a majority price), by group. A price change: the price differs from that of the previous observed month and is still charged in the next observed month, each at most 3 months away. Same price as another clinic of the group: the share of prices that at least one other clinic of the same column charges for the same service in the same month. Chain list price: the modal price of the service in the month among at least 5 of the chain's clinics in the column (for independents, Evidensia's list price), as a share of the service-months with a list price. Cross-clinic SD: the standard deviation across the clinics of the column of the log price of a service in a month, averaged over services and months. *Sample:* the clinics owned by Evidensia or AniCura throughout the data and the never-acquired independent clinics (without Distriktsveterinärerna), without hospitals, January 2019 to March 2025 in every row (the January–February row: 2020–2024; from April 2025, fewer insurers report invoice lines). *Definitions:* Section B.2.

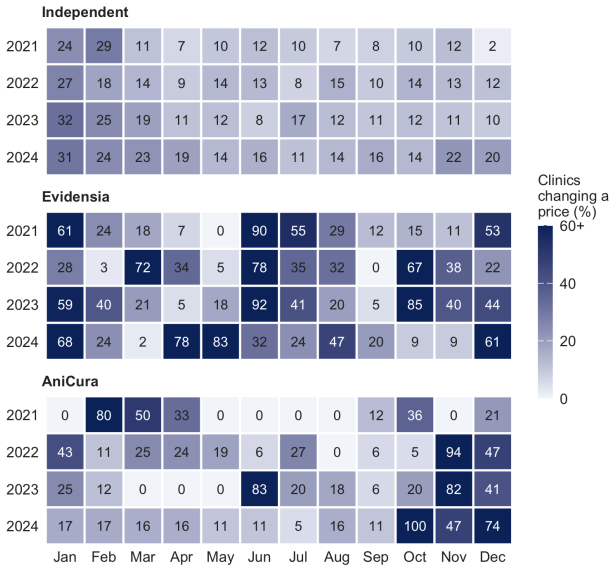


Figure F.1: Share of clinics that change at least one price, by owner, year and month

Notes: Measure: the percentage of clinics that change the price of at least one Main-basket service in the month, among the clinics with at least one price in the month that can be compared with the previous observed month (a price change as in Table F.2); cells with fewer than 5 clinics are blank, and the colour scale is capped at 60%. *Sample:* Evidensia and AniCura, the clinics owned by the chain throughout the data; Independent, the never-acquired clinics (without Distriktsveterinärerna); hospitals are excluded; calendar years 2021–2024 (months to March 2025 are used to confirm changes, Table F.2). *Definitions:* Section B.2.

Table F.3: Prices of competing clinics in the city of an acquisition

	TWFE		CS, never treated		dCDH, never switchers		Clinics		
	Estimate	p-trend	Estimate	p-trend	Estimate	p-placebo	Treated	Controls	N (clinic-years)
Core	-0.048	0.041	-0.064	0.301	0.015	0.054	14	210	1,191
	(0.037)		(0.044)		(0.019)				
Main	-0.051	0.794	-0.081	0.568	-0.018	0.845	17	238	1,400
	(0.036)		(0.068)		(0.027)				
Broad	0.027	0.348	0.012	0.298	0.065**	0.898	17	248	1,570
	(0.033)		(0.068)		(0.030)				

Notes: Measure: the effect of an acquisition on the basket prices of the other clinics in the city, relative to the control clinics; columns and estimators as in Table D.1. *Sample:* clinics of any owner except Svenska Veterinärgruppen; treated, those in a city where one of the 12 acquisitions takes place, dated by the first such acquisition; controls, those in cities without an acquisition; the acquired clinics themselves and clinics without a year before the acquisition are dropped. *Specification:* as in Table D.1. *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

F.1 Centralised pricing and the IT switch

The IT switch is the month in which an acquired clinic starts to bill with its acquirer's invoice texts (Section B.2; Table B.4 gives the month for each clinic). At the switch, the clinic's prices also move onto the chain's price structure: the share of its Main-basket prices that another chain clinic also charges rises from about 50% to above 80% in the quarter of the switch, and reaches the level of the established Evidensia clinics a year later (Figure F.2, panel a). Event studies with never-acquired clinics as controls, and narrower measures that count only invoice texts and list prices particular to a chain, show the same (Figure F.2, panels b and c). Many of the prices the acquired clinics are given are prices the chain charged in earlier

years: the clinics are moved onto the chain's price structure at a lower level and then raised towards it. Figure F.3 and Table F.4 date the effect on prices by the switch, and Figure F.4 follows each clinic's invoice texts and Main-basket price around it.

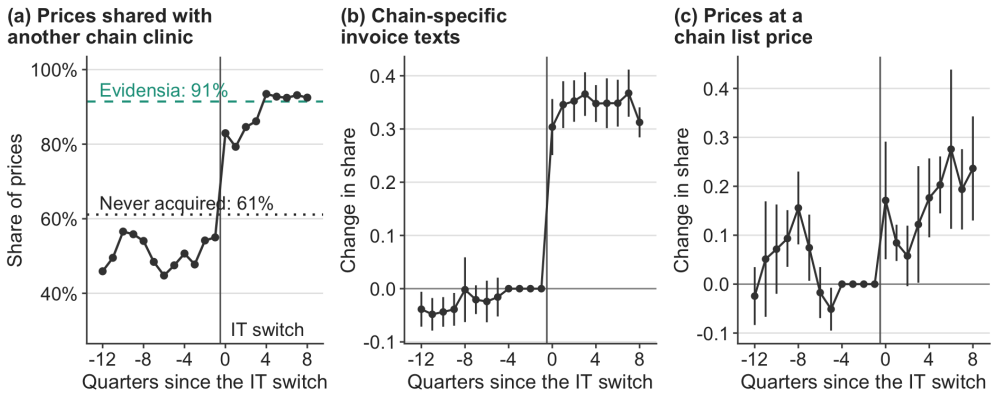
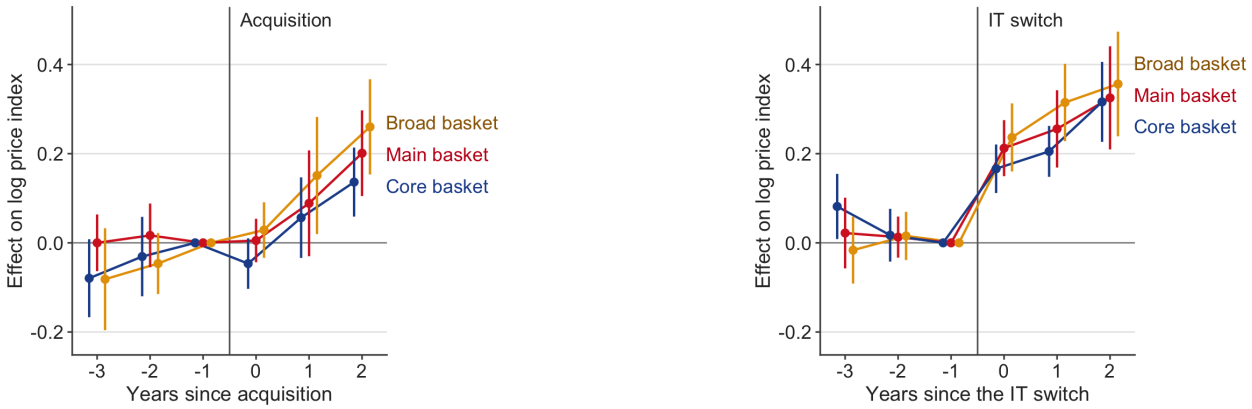


Figure F.2: Around the IT switch: prices shared with another chain clinic, chain-specific invoice texts and chain list prices

Notes: Measure: by quarter since the IT switch, (a) the mean share of the clinic's Main-basket prices (service-months) that equal a price that at least one other clinic of Evidensia or AniCura charges for the same service in any month of the data (the data, not estimates); (b) the share of the clinic's non-emergency invoice lines, from all four insurers, whose text is used by at least 6 Evidensia or 4 AniCura clinics other than the acquired clinics and by at most 5% of the never-acquired independent clinics in the calendar year (a broader measure than the IT-switch rule of Section B.2, which uses one insurer and the acquirer's texts); (c) the share of its Main-basket prices at a chain list price (the modal price of the service in the month among at least 5 clinics of the chain). *Sample:* the 11 acquired clinics with a detected IT switch; in (a), also the clinics owned by Evidensia throughout the data (dashed) and the never-acquired clinics (dotted), each as the mean over the clinic-quarters of 2023–2024; quarters in which fewer than 50% of the acquired clinics are observed are not shown. *Specification:* (b), (c) TWFE event studies with the never-acquired clinics as controls, clinic and quarter fixed effects and quarters −4 to −1 as the reference. *Estimates:* mean effect over quarters 0 to 12, weighted by the acquired clinics observed (standard error): (b) 0.343 (0.014), (c) 0.210 (0.024). *Definitions:* Section B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).



(a) Dated by the closing of the acquisition

(b) Dated by the IT switch

Figure F.3: Effect on prices dated by the closing of the acquisition and by the IT switch (TWFE)

Notes: Measure: the Core, Main and Broad basket price indices, by year since (a) the closing of the acquisition and (b) the IT switch, relative to year −1. *Sample:* the 11 acquired clinics with a detected IT switch. *Specification:* TWFE, as in Figure 1, panel (b). *Estimates:* mean effect over years 0 to 2 (standard error), dated by the closing: Core 0.048 (0.032), Main 0.098 (0.040), Broad 0.147 (0.045); dated by the IT switch: Core 0.226 (0.030), Main 0.262 (0.043), Broad 0.301 (0.046); see also Table F.4. *Definitions:* Section B.2. Bars show 95% confidence intervals (standard errors clustered by clinic).

Table F.4: Effect on prices dated by the IT switch

	TWFE		CS, never treated		dCDH, never switchers		Clinics		
	Estimate	p-trend	Estimate	p-trend	Estimate	p-placebo	Treated	Controls	N (clinic-years)
Core	0.226***	0.074	0.225***	0.182	0.227***	0.106	11	146	675
	(0.030)		(0.035)		(0.024)				
Main	0.262***	0.811	0.270***	0.944	0.266***	0.772	11	177	883
	(0.043)		(0.052)		(0.040)				
Broad	0.301***	0.303	0.302***	0.689	0.303***	0.752	11	186	1,046
	(0.046)		(0.054)		(0.038)				
Spending per visit	0.090	0.496	0.097	0.520	0.094	0.406	11	188	1,162
	(0.072)		(0.080)		(0.063)				

Notes: Measure: the effect on prices and spending with the event dated by the IT switch instead of the closing of the acquisition; estimate: the mean effect over years 0 to 2, since the following year is observed only in part, and only for the clinics that switched first. *Sample:* the 11 of the 12 acquired clinics with a detected IT switch, and the never-acquired clinics. *Specification:* columns and estimators as in Table D.1. *Definitions:* Section B.2 (IT switch). Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

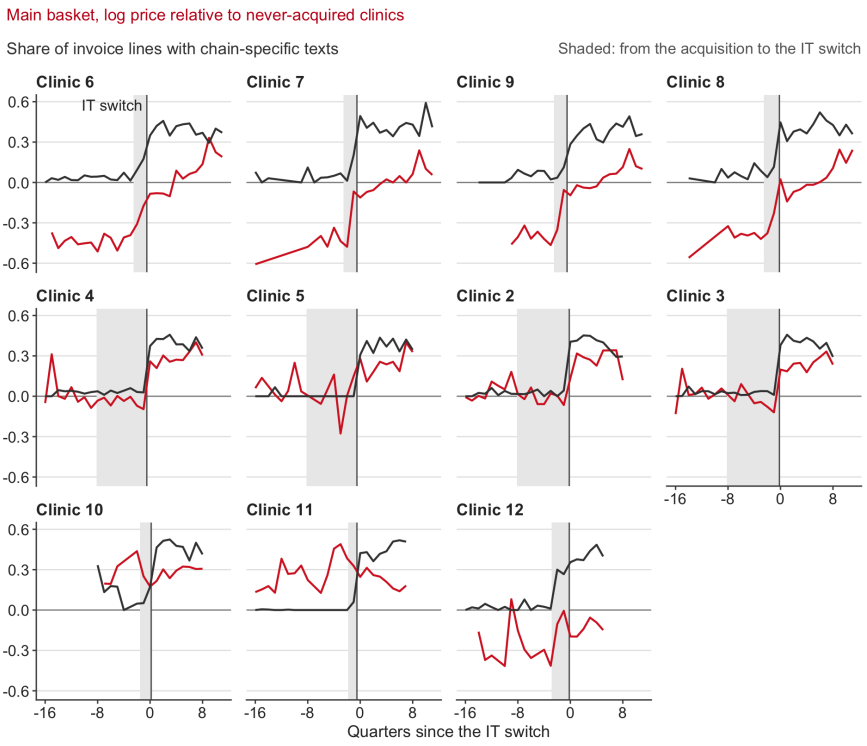


Figure F.4: Clinic by clinic: share of chain-specific invoice texts and the price of the Main basket around the IT switch

Notes: Measure: by quarter, the clinic's Main basket price index relative to the never-acquired clinics (log points; red) and the share of its invoice lines with chain-specific invoice texts as in Figure F.2, panel (b) (dark grey); vertical line: the IT switch (Section B.2), dated on one insurer's lines only; shaded: from the closing of the acquisition to the IT switch. *Sample:* one panel per acquired clinic with a detected IT switch, sorted by the date of the switch (clinics numbered by closing date, Table B.4); the clinic without a detected switch is not shown. *Definitions:* Section B.2.

G Brand value and quality

This section reports the probability of a follow-up visit within 30 days and of prescribing antibiotics, by owner (Tables G.1 and G.2), and pet owners' satisfaction in the UK (Figure G.1; CMA, 2026). Section G.1 defines the customer measures of the paper's Figure 4 and shows them city by city.

Table G.1: Probability of a follow-up visit within 30 days

Outcome (0/1)	(1)	(2)	(3)	(4)	(5)	**(6)**	Mean of outcome
Follow-up at the same clinic	-0.038***	-0.027***	-0.020**	-0.019**	-0.014*	-0.015*	0.258
	(0.012)	(0.010)	(0.009)	(0.009)	(0.008)	(0.008)	
Follow-up within the same chain	-0.008	0.008	0.016*	0.018**	0.022***	0.021***	0.275
	(0.012)	(0.009)	(0.009)	(0.008)	(0.008)	(0.008)	
Year	✓	✓	✓	✓	✓	✓	
Hospital		✓	✓	✓	✓	✓	
Region			✓	✓	✓	✓	
City type				✓	✓	✓	
Diagnosis type					✓	✓	
Breed type, emergency, pet age						✓	
N (visits)	397,894	397,894	397,894	397,894	396,988	396,983	

Notes: Measure: the probability that the visit is followed by another visit of the same animal within 30 days, at the same clinic (first row) or at a clinic of the same chain (second row); mean of outcome: the share of visits with a follow-up in the sample of column (6). A follow-up within the same chain can take place at any of the chain's clinics, while a follow-up of an independent clinic's patient is counted only at that clinic. *Sample:* one observation per visit of the cross-sectional sample with positive spending, including the visits reported without invoice lines; the number of observations differs across columns because observations with a missing control, or alone in a fixed-effect cell, are dropped. *Specification:* linear probability models on a chain dummy (AniCura or Evidensia at the visit; reference: independent clinics), with the fixed effects marked below each column; the preferred specification is in bold. *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

Table G.2: Probability of prescribing antibiotics

Outcome (0/1)	(1)	(2)	(3)	(4)	(5)	**(6)**	Mean of outcome
Antibiotics prescribed	0.018**	0.000	-0.001	0.002	0.002	0.001	0.029
	(0.007)	(0.004)	(0.004)	(0.003)	(0.003)	(0.003)	
Year	✓	✓	✓	✓	✓	✓	
Hospital		✓	✓	✓	✓	✓	
Region			✓	✓	✓	✓	
City type				✓	✓	✓	
Diagnosis type					✓	✓	
Breed type, revisit, emergency, pet age						✓	
N (visits)	337,099	337,099	337,099	337,099	336,209	336,206	

Notes: Measure: the probability that antibiotics are prescribed at the visit; mean of outcome: the share of visits with antibiotics in the sample of column (6). *Sample:* one observation per visit of the cross-sectional sample with invoice lines and positive spending (the visits reported without invoice lines have no outcome); the number of observations differs across columns because observations with a missing control, or alone in a fixed-effect cell, are dropped. *Specification:* linear probability models on a chain dummy (AniCura or Evidensia at the visit; reference: independent clinics), with the fixed effects marked below each column; the preferred specification is in bold. *Definitions:* Section B.2. Standard errors clustered by clinic in parentheses. $^{*}p < 0.1$, $^{**}p < 0.05$, $^{***}p < 0.01$.

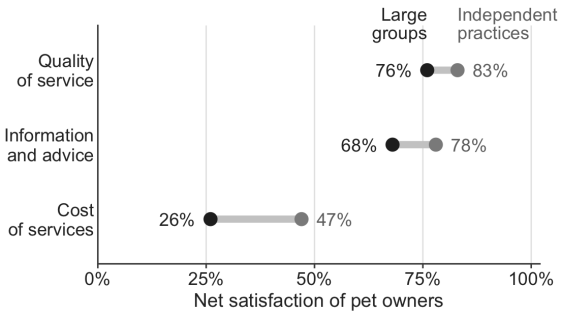


Figure G.1: Pet owners' satisfaction in the UK: independent practices and large veterinary groups

Notes: Measure: net satisfaction of UK pet owners with three aspects of their veterinary practice (the share of respondents who were satisfied minus the share who were dissatisfied), for customers of independent practices and of the six large veterinary groups taken together. *Sample:* the pet owner survey of the Competition and Markets Authority (2,376 respondents, November 2024); the six groups are CVS, IVC Evidensia, Linnaeus, Medivet, Pets at Home and VetPartners (Linnaeus, like AniCura, has been owned by Mars since 2018). *Source:* CMA, Veterinary services for household pets, Summary of final report, p. 9 (paragraphs 27 and 28(a), footnote 11).

G.1 Customers of the acquired clinics

The paper's Figure 4 uses the data of one insurer (Insurer A) only, since the other insurers report the acquired clinics only from the IT switch, and full years only (October to September, up to 2023/24). A city enters if billing coverage cannot drive its measures: the acquired clinic billed Insurer A directly from at least three years before the event (clinics that start billing Insurer A around the event have fewer than half as many visits after it as the clinics kept); the city's other clinics have at least 20 Insurer A visits in year -1; and the acquired clinic has at least 20 customers in year -1. Cities are numbered in the alphabetical order of their names. This leaves Cities 5, 6 and 9 at the acquisition and Cities 3, 5, 6 and 9 at the IT switch.

In panel (a), the share is that of the acquired clinics in the Insurer A visits of the city's clinics with at least 5 such visits in year -1; the clinics are fixed, so a clinic that starts billing Insurer A later does not count. A customer is an insured animal. Panel (b) counts the customers of the previous year who visit the clinic again, and panel (c) the customers who did not visit it in the previous year; both are per 100 customers of the previous year, so they are separate flows and do not sum to 100. The comparison clinics are the never-acquired independent clinics with one third to three times as many customers in the previous year, in the same years, averaged over clinics. Cities are weighted by the acquired clinics' Insurer A visits in year -1 in panel (a) and by their customers in panels (b) and (c). Figure G.2 shows each city.

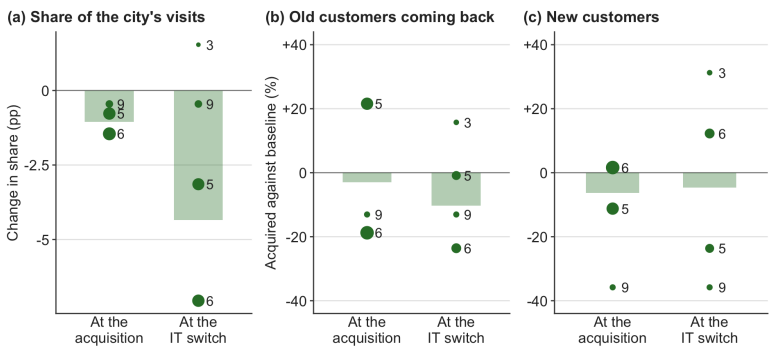


Figure G.2: Customers of the acquired clinics, city by city

Notes: Measure: the measures of Figure 4, city by city: (a) the change in the acquired clinics' share of the city's Insurer A visits from year −1 to the average of years 0 and 1 (percentage points); (b) old customers coming back and (c) new customers, per 100 customers of the previous year, for the acquired clinics against never-acquired independent clinics of a similar size in the same years, in % of the latter. Bars: all cities, weighted by the acquired clinics' size, as in Figure 4; dots: each city, sized by its weight and labelled with its number. *Sample:* 3 cities at the acquisition and 4 at the IT switch; City 9 is acquired and switched in the same year, so it appears with the same values at both events. *Definitions:* Section G.1.

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